COVID-19 Information Treatments Shift Related Policy Preferences and Plans for Consumer Behavior

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Abstract

We investigate the role of information exposure in shaping attitudes and behaviors related to the SARS-CoV-2 (COVID-19) pandemic and whether baseline political affiliation and news diet mediate effects. To do so, we randomly assigned 5,009 U.S. adults to nine brief text-based segments related to the dynamics of the pandemic and the safety of various behaviors. We then estimate how the provision of information affects 15 binary outcomes related to COVID-19 policy preferences, expected consumer behavior, and beliefs about safety. Relative to a reference group, mean effects reach significance (95% CI) in 47 out of 120 baseline models. The mean point estimate for significant effects is 7.4 ppt. The baseline effects are large for all outcomes except beliefs. By contrast, interaction effects by political party and media diet are significant for beliefs but rarely significant for policy and behavioral attitudes. These findings suggest partisan policy and behavioral gaps are driven, at least in part, by exposure to different and often low-quality information, and that equalizing information sources would lead to partisan convergence in beliefs about the safety of COVID.

Keywords: Beliefs; Behavioral Economics; Consumption; COVID-19; Expectations; Information.

JEL Codes: C91, D83, E21, E71, I38

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1 Introduction

There is a large theoretical and empirical literature about the determinants of beliefs and expectations, together with their resulting effects on economic and social activity.[1] However, recent evidence shows that political party affiliation is also an important explanatory variable for policy preferences (Alesina et al., 2020), including those related to the COVID-19 pandemic (Allcott et al., 2021; Bursztyn et al., 2020; Makridis et al., 2021; Cao et al., 2022). Given rising political polarization in the United States (Andris et al., 2015; Mason, 2018), which is characterized by lack of inter-party cooperation among politicians and increasing dislike and distrust of partisan opponents, it is unlikely that a close link between partisan politics and public health beliefs will result in accurate beliefs or optimal policies (Rothwell and Desai, 2020; Makridis et al., 2021).

Misinformed beliefs and self-serving biases can have negative consequences for individuals, firms, and societies (Akerlof and Dickens, 1982; Foster et al., 1984; Babcock and Loewenstein, 1997; Farmer, 1999; Barber and Odean, 2001). If misinformation stems from exposure to heterogeneous information sources, then providing common sources of high-quality information should lower the incidence of erroneous beliefs and encourage more cooperative civic behavior across partisan lines, e.g. with income inequality (Kuziemko et al., 2015; Alesina and Stantcheva, 2018), inflation (Armantier et al., 2016), innovation policy (Hjort et al., 2021), and financial decision-making (Bursztyn Leonardo Ederer et al., 2014). Alternatively, if beliefs are tied to other dimensions identity, then information may either have no effect or be interpreted differently, depending on

[1] Macroeconomists have long suspected a link between expectations and economic activity (Carroll and Dunn, 1997; Carroll, 2003), embedding them more formally in quantitative models (Angeletos and La'O, 2013). These have frequently emphasized the role of personal experience on beliefs through: economic crises (Malmendier and Nagel, 2011), inflation (Malmendier and Nagel, 2016), foreclosures on home mortgages (Makridis and Ohlrogge, 2021), gasoline prices (Binder and Makridis, 2021), the stock market (Adam et al., 2017), and housing prices (Kuchler and Zafar, 2019; Kaplan et al., 2020).

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prior convictions and affiliations. In the context of a pandemic, clarifying these relationships is crucial to promoting scientific and cognitive literacy for optimal decision-making.

To understand the effects of exposure to COVID-19 related information on policy preferences, consumer behavior, and beliefs, we conduct a randomized controlled experiment using the Franklin Templeton-Gallup Economics of Recovery Study containing nationally representative data from 5,009 U.S. adults in early December 2020. Respondents were randomly assigned to one of nine information conditions described and their policy preferences, intentions to engage in public-facing consumption, and more general beliefs about the pandemic were subsequently collected across 15 survey items. We begin by documenting several new facts, including large gaps across political affiliation in policy preferences, consumer behavior, and information sources. For example, 72% of Republicans and 54% of Democrats support in-person elementary and secondary schooling, and Republicans are 21 percentage points (ppt) more likely to say they are likely to eat out within the next 30 days. We also find that Republicans and Independents consume a more politically diverse news diet than Democrats, consistent with literature showing the dominant influence of highly-professional, but politically left-leaning media (Benkler et al., 2018).

Our primary research objective is to estimate the causal effect of providing information on 8 policy preferences, 4 consumer behavior, and 3 beliefs. Our randomly-assigned segments vary in terms of emphasizing alarming news (e.g. record high cases) or more reassuring news (e.g. the low-probability of infection while wearing a mask on a flight). We test the causal effect of 8 distinct treatments (each corresponding to a different information segment) on 15 outcomes. Each of the 15 models in our preferred baseline specification contains 8 tests.

Results show large effects on policy and consumer-related attitudes following the introduction of this randomized information. Our reference group is informed that new daily COVID-19 cases

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have recently reached a record, which was true at the time of the survey. Compared to that group, providing people an alternative piece of information changes the likelihood of engaging in public-facing consumer behavior by 5.4 ppt in our preferred modelling structure. The point estimate is significant (95% CI) in 69% of tests, with a mean of 6.5 ppt when significant. The mean effect on policy preferences is 4.7 ppt, and those are significant in 36% of tests (8.3 ppt mean effect when significant). For example, informing people about the American Academy of Pediatrics (AAP) recommendation of having students learn in-person in the coming year is associated with a 15 ppt rise in the probability of supporting in-person elementary and secondary schooling. Similarly, it has an 11 ppt effect on the probability of eating out within the next month, and an 8 ppt decrease in supporting the closure of non-essential businesses.

Surprisingly, we do not observe significant effects on our belief outcomes, which measure confidence in avoiding infection, the belief that COVID-19 is under control in the respondent's state, and plans to take the forthcoming COVID-19 vaccination. The mean effect is just 2.1 ppt and the results are significant in only 8% of tests. While our treatments did not directly address these issues, we predicted that reassuring information would boost confidence and alarming information would decrease feelings that the pandemic is under control.

We mostly reject the hypothesis that these effects on consumer behavior and policy vary by political orientation or news diet. For consumer behavior, 0% of our tests for different effect for Democrats compared to Republicans are significant and 13% when we test a liberal-leaning diet against a right-leaning diet. For policy outcomes, only 16% are significant for partisan interaction effects and 23% for media diet interactions. On the other hand, we find heterogeneous effects of information on belief outcomes. 25% of tests reach significance when we examine interactions between party or news diet and beliefs. These results show consistent evidence that Democrats

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gain additional confidence and a greater sense of control relative to Republicans when exposed to our more reassuring news segments.

We interpret these findings as consistent with a theoretical framing that allows for attitudes to emerge based on the cumulative exposure to information sources. Partisanship and a partisan news diet provide filtering mechanisms that alter information exposure. New information alters attitudes. We find very weak evidence, however, that partisanship and prior news consumption distorts the interpretation of new information, at least in the context of a novel social phenomenon like the COVID-19 pandemic.

Our paper makes two primary contributions. The first is to identify precise measures of how much information affects self-reported policy-preferences and other attitudes collected immediately after exposure. More broadly, these causal estimates help clarify how cultural or partisan gaps in attitudes, behavior, and policy preferences may emerge as a result of information exposure. Our analysis of partisanship and the news diet distinguishes our paper from many information experiments in the literature, many of which are reviewed by Haaland and Roth (2022). In relevant work, Faia et al. (2021) studies how partisan views interact with information selection and processing, leading media consumers to filter news that complements their prior convictions. The self-selection of information can gradually coalesce into biased but firmly-held attitudes, making exposure to countervailing information important in truth discovery. Coppock and Kirby (2018) randomly assigned participants to read opinion pieces and found long-lasting effects on beliefs that were similar across political parties. Likewise, Nyhan and Reifler (2019) find that information treatments reduce mistaken beliefs and that the effects do not strongly depend on prior attitudes. Haaland and Roth (2021) and Abascal et al. (2021) found that information led to partisan convergence in beliefs but not policy positions on the topics of race and immigration,

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respectively. Whether new information has similar or heterogeneous effects on beliefs is crucial to theories dealing with the "polarization of reality" (Alesina et al., 2020). We also provide relevant evidence on the pandemic, which was not initially polarized because it was a novel social context.

Secondly, our paper also builds on a number of recent contributions over the COVID-19 pandemic and provides important context to literature on optimal public health policy and the public's understanding of official recommendations and compliance with them. As has been well-established, many adults are misinformed about COVID-19 (Alsan et al., 2020) and there are notable partisan gaps in attitudes, policies, and behaviors (Allcott et al., 2021; Makridis et al., 2021). Atkesson et al. (2020) finds that individuals dramatically overestimate the infectiousness of COVID-19 and that providing people with expert analysis partially corrects their beliefs, with important mitigation-related behavioral implications. We validate these results and extend them to policy preferences and consumer behaviors. Likewise, in an experimental setting, (Torres et al., 2021a) show that physician-delivered information increased COVID-19 knowledge, information-seeking, and self-reported protective behaviors. Our paper also builds on important non-experimental work that documents how exposure to alarming versus reassuring news segments affected social distancing behavior early in the pandemic (Bursztyn et al., 2020).[2] Understanding the role of information in explaining misinformed beliefs about COVID-19 is important for identifying paths forward for an economic and social recovery, particularly due to the scarring effect of beliefs on economic activity (Kozlowski and Veldkamp, 2020; Kozlowski et al., 2020).

[2]Other notable papers include: Ash et al. (2020), Simonov et al. (2020), Breza et al. (2021), and Torres et al. (2021b).

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2 Methods

2.1 Experimental Design

In December of 2020, we launched an experiment using the Franklin Templeton-Gallup Economics of Recovery Study containing nationally representative data from 5,009 U.S. adults. Respondents were randomly assigned to one of nine conditions described in Table 1. These were akin to news or information that respondents may have seen as media headlines, shared social media content, or summary messages from radio or television segments.

The research team purposely chose segments that conveyed more or less alarming information, while requiring the segment to be accurate, fact-based, and objectively stated. In this sense, the goal of the experiment was not to precisely test actual media content, which can often be hyperbolic, but test the effects of high-quality information that could be readily sourced and confirmed as originating from credible public agencies or experts.

For example, some respondents view the statement that the "American Academy of Pediatrics (AAP) strongly advocates that all policy considerations for the coming school year should start with a goal of having students physically present in school. The importance of in-person learning is well-documented and there is evidence of the negative impacts on children because of school closures in the spring of 2020." However, others view the statement "there are currently 77,000 Americans in the hospital with the coronavirus. That is higher than the previous peak of 60,000 hospitalizations in April." We also treat some with economic data–which may raise the salience of respondent concerns about how COVID-policies have harmed the economy–without an explicit public health context: "according to the U.S. Bureau of Labor Statistics, the unemployment rate

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in October is 6.9%."

Following our experiment, our survey collected data on 15 outcomes. This survey structure effectively constrained our analysis, since items collected before the experiment could not have been affected by the experiment. To ensure that we are fully comprehensive and not cherry-picking results, we report coefficients from all 15 items. We group them into three categories: 1) 8 policy preferences 2) 4 consumer behavior 3) and 3 beliefs (see Figure 1). Policy preferences are captured as support or lack thereof for policies that would require mask use in public, place restrictions on in-person schooling, social gatherings, restaurants, and bars. Consumer behaviors related to plans within the next 30 days to book a reservation at a restaurant, a flight, or hotel accommodations. Beliefs capture general level of confidence in avoiding COVID-19 infection while out in public, whether the virus is under control in the respondent's state, and willingness to get the soon-to-be-released COVID-19 vaccine. The latter is, admittedly, not easily categorized with the others, as it involves public-health behavior, informed by beliefs about the vaccination process, efficacy, and safety. Analytically, it is convenient to group those three because our treatments do not provide direct information about vaccines or the other two "belief" items, so our expectation is that our treatments will have a weaker effect on those outcomes.

Nine of our 15 outcome variables were collected as a binary response, and we coded the other six accordingly. For consumer behavior, responses are coded as 1 (for likely) if the respondent indicated either of the top two choices (somewhat likely or highly likely) and 0 otherwise. Respondents were coded as 1 if they were "very confident" they could protect themselves from COVID-19 infection while out in public on a 1-4 scale and 0 otherwise. Those who would agree to be vaccinated with the forthcoming COVID vaccine were coded as a 1, while those who replied "no" or "I don't know" were coded as 0. Tables 1 and 2 in the SI present descriptive statistics on the

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incidence of each of these information treatments across the different outcome variables, which provides guidance for interpreting the marginal effects in Section 2.

While randomization alleviates concerns of omitted variables bias, it does not completely eliminate the threat that experimental groups differ based on their predilections to behave or believe certain things about COVID. For each respondent, we observe a wide array of characteristics, including: binary variables for race and ethnicity, age, age-squared, and a cubic value, a binary variable for male sex, binary variables for levels of educational attainment, whether the respondent is a homeowner, political party affiliation, current employment status, household income (11 bins of fixed effects), political affiliation, pre-COVID travel behavior, pre-COVID indebtedness versus saving, and self-reported medical risk factors for oneself and family members. We also include the results of a two-item numeracy test, because numeracy may be related to the ability to correctly interpret scientific guidelines and translate risk probabilities into appropriate behavior and attitudes. The discussion section of the supplement provides details about how these data were collected and coded.

To test of whether lack of balance may bias the interpretation of mean differences on our outcomes by experimental group, we regressed assignment status for each of our 9 experimental groups on a vector of the explanatory variables listed above and calculated the predicted values. The predicted values captures the probability that an individual with a given set of characteristics will be assigned randomly to one of the nine groups. Each respondent has nine such values. In a second set of regressions, we regress each of our 15 outcomes on demographically-predicted assignment. Significant values would suggest bias in assignment. The results are reassuring that our randomization strategy mitigated bias with respect to our outcomes of interest. Since there are 9 experimental groups and 15 outcomes, there are 135 coefficients to test. Of these 135, only

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4 (3%) were significant at 95% confidence intervals. In other words, random chance generated clusters of people with demographic characteristics that make them slightly more inclined toward an outcome of interest in only 3% of instances. These results are available upon request.[3]

Since we could not completely rule out lack of balance in our randomization, our preferred models include our full list of demographic controls. We also attempt to rule out other sources of bias, such as local exposure risk, from unusually high or low deaths per capita or population density, using country level data. Supplemental Figure 1 shows how our preferred model performs against two alternative models: one with zero controls and one with additional controls for country-level risk. The results are broadly similar, though our baseline model enhances levels of significance for the consumer behaviors in some cases, largely by controlling for pre-COVID travel patterns that are highly correlated with consumer behavior during the pandemic.

2.2 Empirical Specification

We exploit the random assignment to experimental conditions to estimate the causal effect of information on these policy preferences and consumer behaviors through the following:

$$y_i = \gamma INFO_i + \phi POL_i + \xi(INFO_i \times POL_i) + \beta X_i + \epsilon_i \tag{1}$$

where y denotes our outcome of interest for person i, INFO denotes our information treatment,

[3]Our survey was approved by Gallup's Internal Review Board, but we did not pre-register the experiment because it was designed early in the pandemic when time was not at a premium and the focus was on generating actionable insights. Nonetheless, to promote replicability, our analysis includes all possible outcomes, and we show results with and without our list of extensive demographic controls (see Supplemental Figure 1 and Supplemental Figure 3). We also make all data and code publicly available.

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POL denotes a vector for political affiliation (Republican or Democrat, normalized to moderates), and X denotes our vector of individual demographic characteristics, and ϵ is our error.

While random assignment reduces the correlation between the error term and our demographic variables (in X), random imbalances between groups can emerge, as discussed above; thus, we include the X terms to guard against this potential bias. We focus on the γ coefficients since these represent the effects of information. In an extended model, we also analyze interaction effects between political affiliation and the treatment, which are captured through ξ. These effects are omitted in our baseline. Significant effects on the interaction term would suggest that treatment effects are heterogeneous across party affiliation, which could lead to belief convergence if the sign is the opposite of the effects captured for the same party through ϕ. We also test models that replace the Democratic Party term and its interaction with whether or not the respondent is coded as having a left-leaning media diet. The results are similar to using partisanship as the interaction and are available in the Supplement (Supplemental Table 5 and Supplemental Figure 2).

Our experimental design does not have a natural control group. We could have omitted the news segment item for a random sub-group of respondents, but we wanted to study as many different segments as possible. For the purposes of this analysis, it is arbitrary which experimental group we omit as the reference group, so we chose a segment that is both representative of popular news and sufficiently different in tone from our more reassuring pieces of information to maximize the contrast. Effects are mean differences from respondents assigned to this group. The segment reads: "November 13th saw a daily record for number of new confirmed coronavirus cases in the United States at 171,376." This piece of information was very close to actual headlines published around the middle of November in popular center-left sources, including: ABC News (2020), Washington Post (2020), and New York Times (2020). Yet, conservative media listeners consumed

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a very different analysis. Transcripts from Fox News hosts Tucker Carlson (Fox News, 2020b) and Greg Gutfeld (Fox News, 2020a) emphasized the undesirability of state-mandated COVID-19 restrictions and suggested the disease burden of COVID-19 had been exaggerated by Democrats.

3 Results

3.1 Descriptive Statistics on Policy Preferences and Information Exposure

Figure 1 (Panel A) documents substantial heterogeneity in policy preferences, beliefs, and consumer behavior by political affiliation. In particular, the mean absolute value gap between Democrats and Republicans is 16 ppt and ranges from 1 to 32. Gaps between Independents and Democrats are smaller (6 ppt) than gaps between Independents and Republicans (12 ppt).The starkest contrasts are over policy preferences: 37% of Democrats and 68% of Republicans support allowing indoor dining, whereas 65% of Democrats and 38% of Republicans support temporarily closing non-essential local businesses. We also see large differences in support for in-person schooling and the likelihood of eating out. Interestingly, other consumer behavior, such as the likelihood of booking hotel accommodations or a flight personal travel are not as far apart (8 and 6 ppt gaps), while the likelihood of booking a flight for business travel is essentially the same (1 ppt difference).[4]

[4]One limitation of our analysis is that it is static, so we cannot study how these partisan differences have varied over the course of the pandemic. For example, Makridis et al. (2021) use panel data to document increased divergence of beliefs about COVID-19 since 2020, even after controlling for demographics and local exposure to COVID cases. In Section 4, we discuss this limitation and leave the dynamic effects to future research, focusing here instead on recovering causal effects from the information treatment.

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One explanation behind the heterogeneity in policy preferences is a difference in the underlying tastes across individuals. Another is exposure to different information. Because of the way that we have designed our information intervention, we are able to rule out heterogeneity in preferences and focus on the contribution of information. To provide some motivation on the exposure to different information, Panel B of Figure 1 examines the composition of individuals' media diet by exploring the type of news outlets that they pay attention to (see the Discussion section of the supplement for an explanation of how we categorize news sources by political orientation based on the political views of its consumers, not the content provided). As expected, most Republicans consume a news diet from sources that are largely consumed by other Republicans and vice versa for Democrats. For example, 53% of Democrats consumer a liberal-leaning diet, and 56% of Republicans consumer a right-leaning diet. Independents fall in-between. Importantly, less than one out of five Republicans and Democrats consume a news diet that consists of sources commonly viewed by the opposite party. Still, a mixed diet is common for roughly one third of our sample. This is consistent with empirical research showing that many Republicans consume center-left news sources (Benkler et al., 2018).

3.2 Effects of an Information Intervention

We estimate Equation 1 for 15 outcome variables and each of the 8 information interventions, relative to the omitted group of record high cases. Starting with Figure 2, we find that there are large causal effects of information on most of our policy outcomes and consumer behaviors. For example, individuals who receive the AAP recommendation are nearly 15pp more likely to support in-person schooling, 13pp more likely to support in-person college, 7pp more likely to

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support indoor dining, and 10pp more likely to support indoor bars (significant at the 5% level).[5]

We find similarly large and intuitive effects for many of our other information treatments. For example, providing information on the low risk of transmission for children or analysis on the low risk of school reopenings is associated with a 10-15pp rise in the probability of supporting in-person schooling and in-person college. However, the effects on support for indoor dining are somewhat smaller. The varying treatment effects are a validation test: if respondents were simply reacting to some omitted factor, they would react to the information interventions similarly.

Turning towards consumer behavior, we see fairly meaningful effects of the information on the AAP recommendation, analysis of low school risks, jobless rate, safe travel case study, and facts on peak death on the probability of eating out, booking a personal flight, traveling for business, and booking accommodations, averaging roughly 5-10 ppt. These effects are intuitive. For example, information on the jobless rate puts coronavirus cases in context of the economic reality that the country was in as of December 2020. Further, the AAP recommendation, safe travel case study, and analysis of low school transmission risk all help mitigate fear about coronavirus by highlight that it is not as easy to catch as the media often made it out to be. Finally, information on peak deaths puts the numbers in context of the crisis in April 2020.

Finally, we turn towards policy preferences and beliefs about the pandemic. In general, we find fairly null associations with mask requirements in public and requirements for workers to wear masks. While people generally become more supportive of requiring workers to wear masks, they become less supportive of mask requirements in public. In both cases, however, we fail to

[5]Supplemental Tables 1 and 2 present mean results by experimental group for all respondents and by political party, respectively. Supplemental Tables 3, 4, and 5 provide our detailed regression results using all the relevant demographic characteristics in our baseline model, party interaction model, and news diet interaction model. Supplemental Figures 2 and 3 show the results from the diet interaction model and the baseline model with no controls. They are directly comparable to Figures 2 and 3 in the main text. We omit all of these from the main text to focus the discussion on what we regard as the most important findings.

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reject the null of no effect. Similarly, we find null effects on whether respondents believe that the coronavirus is under control in their state, whether they are safe from infection, and whether they would accept the vaccine. Both the pro masking case study and AAP recommendation make people roughly 5 ppt less likely to accept a hypothetical vaccine.

Next, Figure 3 potential heterogeneity by political affiliation. For ease of interpretation, we report the interaction effect between an indicator for identifying as Democrat, normalized to identifying as Republican; we drop Independents. Importantly, these coefficients are not the mean effects—they are the interaction effects, i.e. ξ from Equation 1. In general, in nearly all cases, we fail to reject the null that there is no treatment effect of the information on policy preferences, consumer behavior, or beliefs about coronavirus. That is important since it suggests the potential for convergence—that is, a similar response among Democrats and Republicans when presented with information in a controlled and less polarized setting. While some of the point estimates are economically meaningful, the confidence intervals can be fairly large, so we recognize that there could be other dimensions of heterogeneity that are also important.

To facilitate the interpretation of all these results, Figure 4 summarizes the results from a meta analysis that presents the coefficients obtained from 120 treatment effects (8 treatments for 15 outcomes) in our baseline model and in two interaction models for a total of 360 coefficients. Panel A provides the point estimates across all 360 tests and Panel B reports the percentage of these tests that reach significance thresholds. Furthermore, Panel C aggregates the percent significant by treatment and Panel D aggregates the percent treatment by outcome. In this sense, Figure 4 allows readers to see which treatments were the most consistently effective. Treatments that included re-assuring expert analysis and not just facts had the largest effects, though facts were also significant in many cases. As for outcomes, expected consumer behavior was the most consistently

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affected by our treatments, suggesting that COVID-19 related information has important real world implications on the economy and on social contact.

Consistent with our earlier results in Figure 3, the meta analysis shows that most of our interaction effects were not significant with the exception of those pertaining to two of our "belief" items: general confidence in avoiding infection and the assertion that COVID-19 is under control in the respondent's state. Democrats and those with a left-leaning news diet displayed significantly larger effects than Republicans or those with a right-leaning news diet.

4 Discussion

4.1 The Value of Information

There is an active debate, perhaps now more than ever, about the causes and remedies to divergence in beliefs, especially in an era with such a large quantity of content proliferating the web and social media. While there is a general understanding that heterogeneity in individual preferences—that is, time-invariant characteristics and tastes—are important determinants of policy preferences and consumer behavior, there is much less agreement on the causal effect of information, particularly over the coronavirus. Our paper reports the results from a novel field experiment conducted within the first year of the COVID-19 pandemic to answer these questions.

We find that short fact-based information segments about COVID-19 have large causal effects on policy preferences and consumer behavior. These effects are largely consistent across political parties and media diets. When it comes to beliefs about the safety of being in public and the extent to which the pandemic is under control locally, respondents were largely unaffected by our

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information segments, but Democrats and those with a left-leaning media diet showed a stronger response than Republicans and those with a right-leaning diet, resulting in belief convergence.

High-quality information is costly to obtain and interpret correctly. In the the Supplementary Materials, we provide a simple theoretical discussion of a model where consumers drift apart and remain on islands of partial knowledge in part because the price and quality of information is hard to discover and evaluate. Our model is consistent with this theoretical framing and suggests that repeated exposure to different information could explain the observed patterns in attitudes.

Given the different media environments documented here (and the literature discussed above), we assume that Democrats and Republicans have been exposed to different information relevant to COVID-19. However, the effects of information seem to be largely independent across groups with heterogeneous baseline beliefs, but the effects are not enough to eliminate differences between partisan groups (see Figures 2 and 3 in the SI). That suggests eliminating partisan polarization in beliefs and behaviors would take long-term exposure to the same sources of information, though we can not provide direct evidence for this in our short-term experiment.

Our rejection of interaction effects (between partisanship or news diet and our experimental treatments) is consistent with previous work that partisans update their beliefs when presented with relevant facts and that psychologically affirmative statements have no additional effects (Nyhan and Reifler, 2019). In this case, partisans update beliefs even when it goes against the norms of their fellow partisans. One explanation, and potential contrast with other information experiments on viruses (Carey et al., 2020), stems from the way our intervention was implemented: we provided succinct facts largely separated from some of the politicization that has been taking place over the pandemic (Makridis et al., 2021). Such politicization, especially in the presence of network effects (Turetsky et al., 2020), can heavily affect beliefs.

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It is also possible that salience and attention matter more than the cumulative effects of prior exposure. If so, our respondents may briefly update their views because of the information we put in front of them without deeply considering the implications or applying them going forward. A limitation of our research is that we do not follow participants over time to see how long these effects last. Yet, using similar techniques, Coppock and Kirby (2018) finds evidence of long-lasting effects from the random assignment of longer texts. Another limitation is that we do not directly reserve consumer behavior, only self-reported intentions to engage in various behaviors. Randomly assigning people to different media diets over a long period of time and observing real-world behavior–such as restaurant outings–in addition to policy preferences and beliefs would be a more compelling and comprehensive test of the power of information, albeit difficult to implement.

4.2 Context and Limitations

Given the massive economic and social harm inflicted by the COVID-19 pandemic and resulting policies, our results underscore that access to high-quality information may have first-order aggregate effects. In fact, such information not only influences the policies that are likely to get adopted (e.g., severity and duration), but also the degree of fear, and therefore, consumer behavior that follows, impacting economic activity. On one hand, exaggerating the risk will lead to too little economic activity to sustain the standard quality of life, as well as the additional effects on mental health and well-being. On the other hand, understating the risk could lead to worse hygiene and sloppy behavior that accelerates the transmission of the virus. Our work, therefore, behaves as a catalyst for achieving the right information that balances between both sides of the spectrum.

Our research nonetheless contains several limitations. First, we only fielded a single study

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that is a snapshot of a moment in time. It is possible, for example, that the value of information has evolved over time and that it is less useful in even more politicized environments. In future work, we plan to field a new survey that traces out the behaviors and beliefs of respondents over time. Second, while we asked about a respondent's media diet, we would like to explore the effects of information in more natural settings and how the source of the information influences beliefs. Especially in an era of social media, we would like to test how intensity of social media use and source exposure influences beliefs and behavior. Third, we would like to understand more about the external validity of these results in other contexts beyond the coronavirus and in other cultural and geographic contexts outside the United States. In particular, different countries may place a different premium on information. We leave these questions for future research.

5 Conclusion

There is a large literature on heterogeneity in beliefs and how they ultimately affect economic activity. There is also growing evidence that access to information is a potential mechanism behind these differences in beliefs—often mediated by personal experience, which is a more salient way of forming expectations. Given the severity and duration of the COVID-19 pandemic and associated government restrictions, a large question remains over the effects of enduring beliefs on economic activity and how best to understand and correct misinformed beliefs.

In this paper, we introduce and report the results of a large-scale randomized controlled experiment with the provision of information about various COVID-19 related facts. First, we document large gaps across political affiliation in policy preferences, consumer behavior, and information sources. In our primary analysis, we estimate the causal effect of information segments

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on policy preferences, consumer behavior, and related beliefs. For example, when our baseline group is informed that new daily cases have recently reached a record—which was true at the time of the survey—informing people about the American Academy of Pediatrics (AAP) recommendation of having students learn in-person in the coming year is associated with a 10pp decline in the probability of supporting the closure of businesses and a 15pp rise in the probability of supporting in-person schooling. Similarly, we find an 11pp rise in the probability of intent to eat out and a 5pp rise in the probability of intent to book a flight. These differences in political opinion and consumption plans are enough to tip elections and dramatically alter local and national economic performance. Our analysis also considers how the baseline results are affected when one varies assumptions about controls, outcomes, and interactions with baseline political affiliation and news diet consumption. We have summarized hundreds of treatment effects and make the data and replication code publicly available for other scholars.

Our results provide, to our knowledge, some of the first causal evidence on the effects of information on both policy preferences and intended consumer behavior. The results are particularly novel and meaningful given that they involve pandemic related policies–which are highly contentious and salient–and consumer behavior relevant to activities that have seen depressed revenue in the United States and globally as a result of disease suppression efforts. In this sense, our results help clarify how public debate and media coverage about COVID-19 has contributed to policy preferences and consumer behavior. Additionally, we study the role of partisanship and news diet in potentially generating heterogeneous treatment effects and conclude that, for the most part, adults respond similarly to new high-quality information, despite partisan-based disparity in baseline beliefs. The exception to this finding is that reassuring information brings Democrats closer to Republicans in terms of how they understand their personal risk and whether

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the pandemic is under control locally.

Nonetheless, our paper leaves several areas for further research. First, do individuals who update their beliefs in response to high-quality information maintain the more accurate beliefs, or does accuracy degrade over time and at what speed? A related question is whether information treatments affect the self-selection of subsequent information. Understanding how to empower consumers to seek out high-quality information is an important implication of this work, and on the supply side, it is also important to better understand how high-quality information can be produced and distributed, especially as it relates to public health information during a pandemic. A secondary series of issues arises as to how more informed individuals might interact with and potentially persuade others on social media and in-person. Finally, at an aggregate level, how long does it take for beliefs to converge or diverge under different plausible scenarios?

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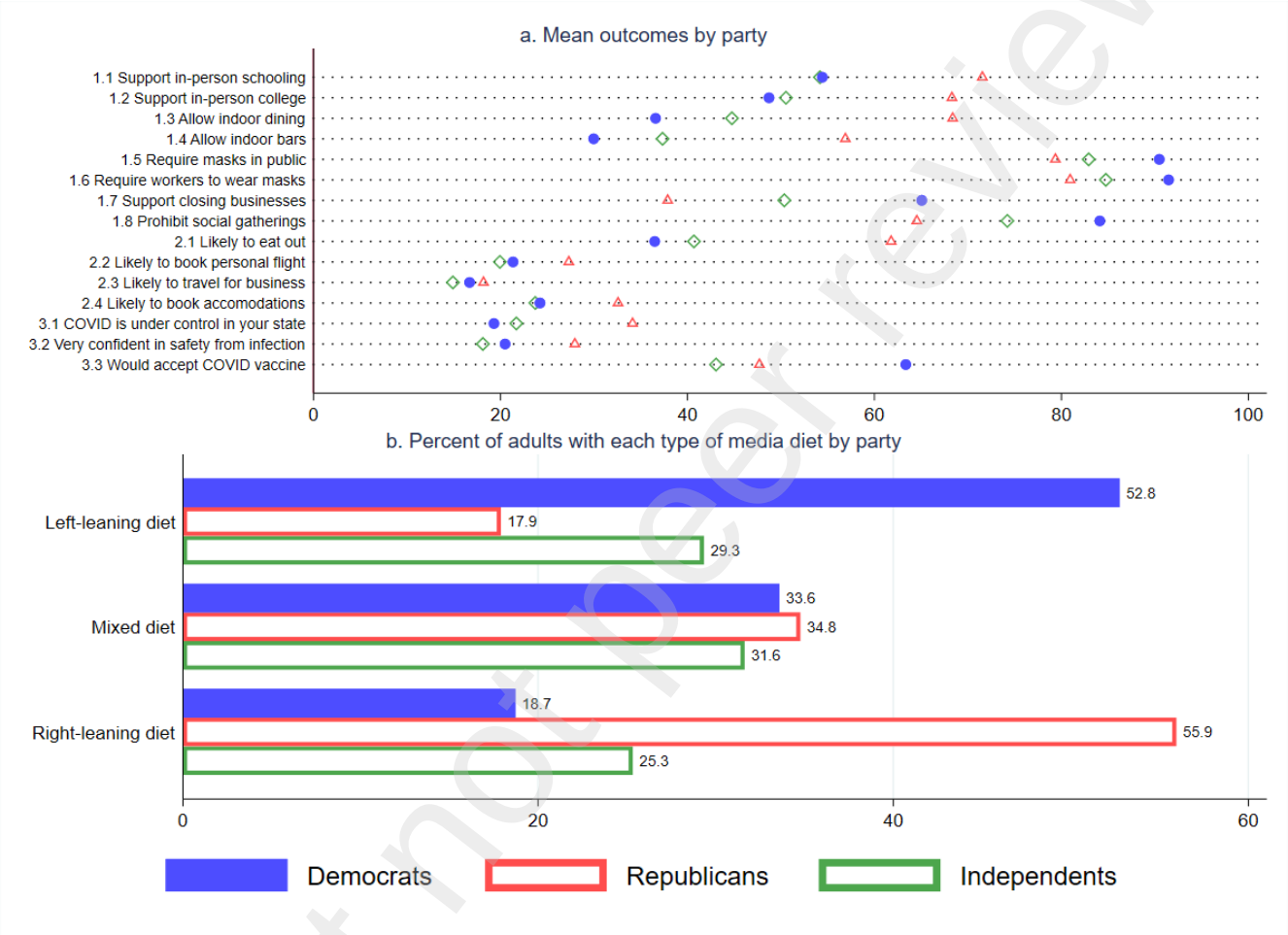
Tables and Figures

Figure 1: Heterogeneity in COVID-19 Attitudes and News Diet, by Political Affiliation



Notes.–Sources: Franklin-Templeton Gallup Economics of Recovery Study, fielded between December 1-7, 2020 with 5,009 U.S. adults responding. Panel a) reports the sample-weighted summary response to 15 outcomes coded on a binary scale by respondent political orientation. Numeric label distinguishes between 1) policies 2) consumer behavior and 3) beliefs. Independents include those who favor no party or favor a third party. Responses are weighted to be nationally representative. Panel b) reports the shares of Republicans, Independents, and Democrats who consume left-leaning, right-leaning, and mixed media diets based on the sources identified in their survey responses. See appendix for further classification details.

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Table 1: Summary of Randomly Assigned Information Treatments

1. The American Academy of Pediatricsâa leading group of physicians who guide health policy regarding childrenâhas issued the following statement: "The AAP strongly advocates that all policy considerations for the coming school year should start with a goal of having students physically present in school. The importance of in-person learning is well-documented, and there is already evidence of the negative impacts on children because of school closures in the spring of 2020."

2. As of October 31, 2020, the Centers for Disease Control (CDC) reports that there have been 192 deaths from the coronavirus for U.S. children aged 14 and younger,which accounts for 0.09% of all deaths from the virus.

3. According to data from the Centers for Disease Control (CDC), 2,715 Americans died per day from the coronavirus in the seven-day period ending April 22. Since then, the most deaths over a seven day period occurred on August 10, averaging 1,154 per day. The week ending November 10th saw 957 deaths per day.

4. November 13th saw a daily record for number of new confirmed coronavirus cases in the United States at 171,376.

5. Economist Emily Oster of Brown University has analyzed coronavirus data from a large number of schools and counties across the country. As of early October, the number of confirmed cases per day was 7 for every 100,000 students and 14 for every 100,000 teachers.These numbers are similar to rates in the neighboring county. Dr. Osterâs analysis suggests that schools are not major sources of COVID-19 spread.

6. In Missouri, two coronavirus-infected hair stylists saw 139 clients. None of the clients developed symptoms, and of the 67 tested, all tested negative for the coronavirus. Both stylists and clients wore masks throughout the encounter. CDC officials concluded that masks prevented the spread of the virus.

7. In a flight from Italy to South Korea in April, 6 passengers unknowingly had the coronavirus, but they infected only one of 299 passengers–a woman who likely used a bathroom after an infected person, according to a report from South Korea's Centers for Disease Control. All passengers were wearing masks.

8. There are currently 77,000 Americans in the hospital with the coronavirus. That is higher than the previous peak of 60,000 hospitalizations in April.

9. According to the U.S. Bureau of Labor Statistics, the unemployment rate in October is 6.9%.

Notes.–The table reports the different information treatments that were contained in the information intervention launched between Gallup and the Franklin-Templeton Foundation.

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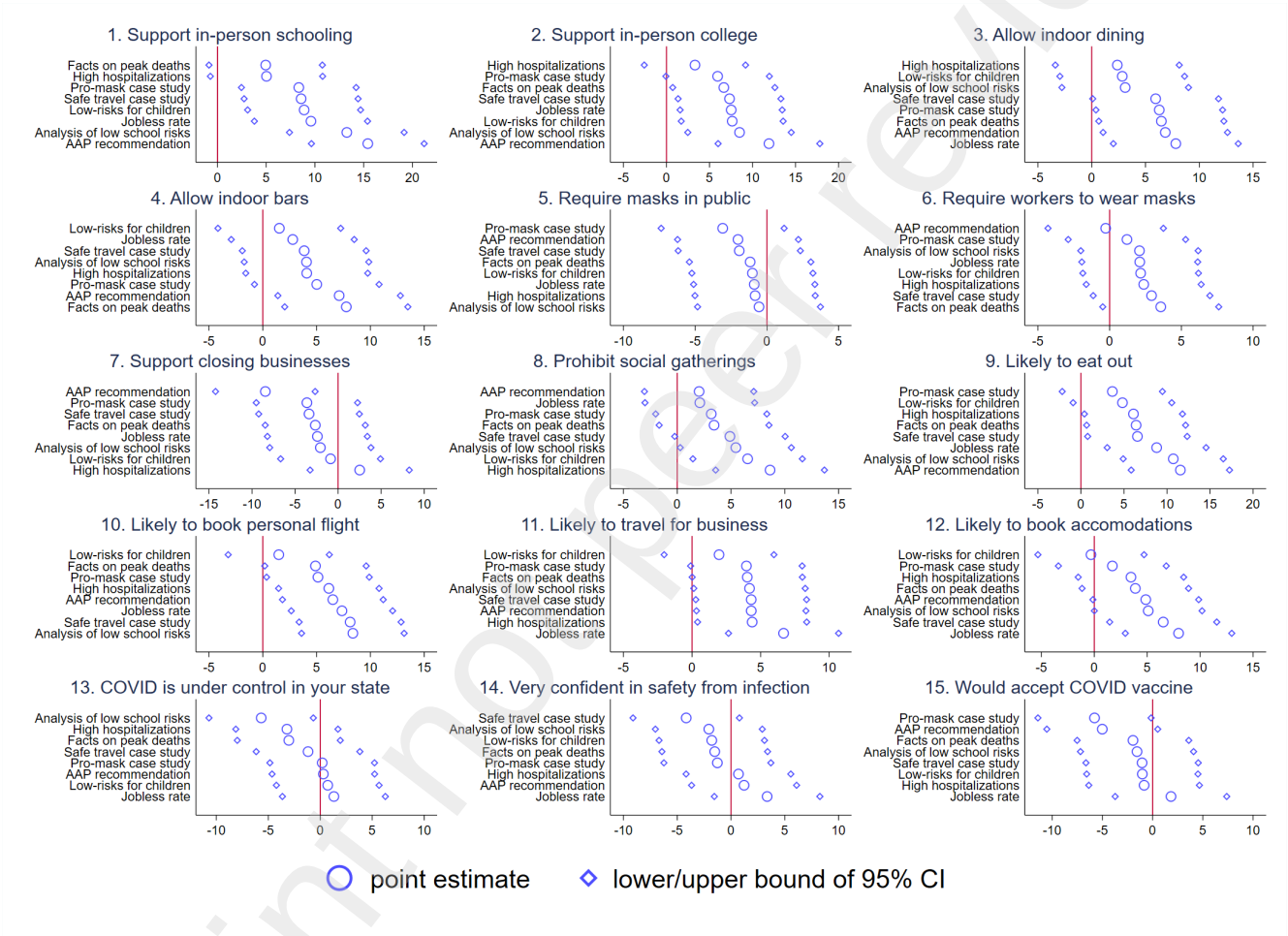
Figure 2: The Causal Effect of COVID-19 Information on Policy Preferences, Consumer Behavior, and Beliefs



Notes.–Each figure plots the coefficients from the baseline model associated with 8 treatments relative to the reference group. Models 1-8 are policy preferences. Models 9-12 are consumer behaviors, and 13-15 are beliefs. All models control for the demographic characteristics listed in text, including: age, education, race, employment status, political affiliation, numeracy, medical risk factors, household income, health status, and gender. 95% confidence intervals are shown.

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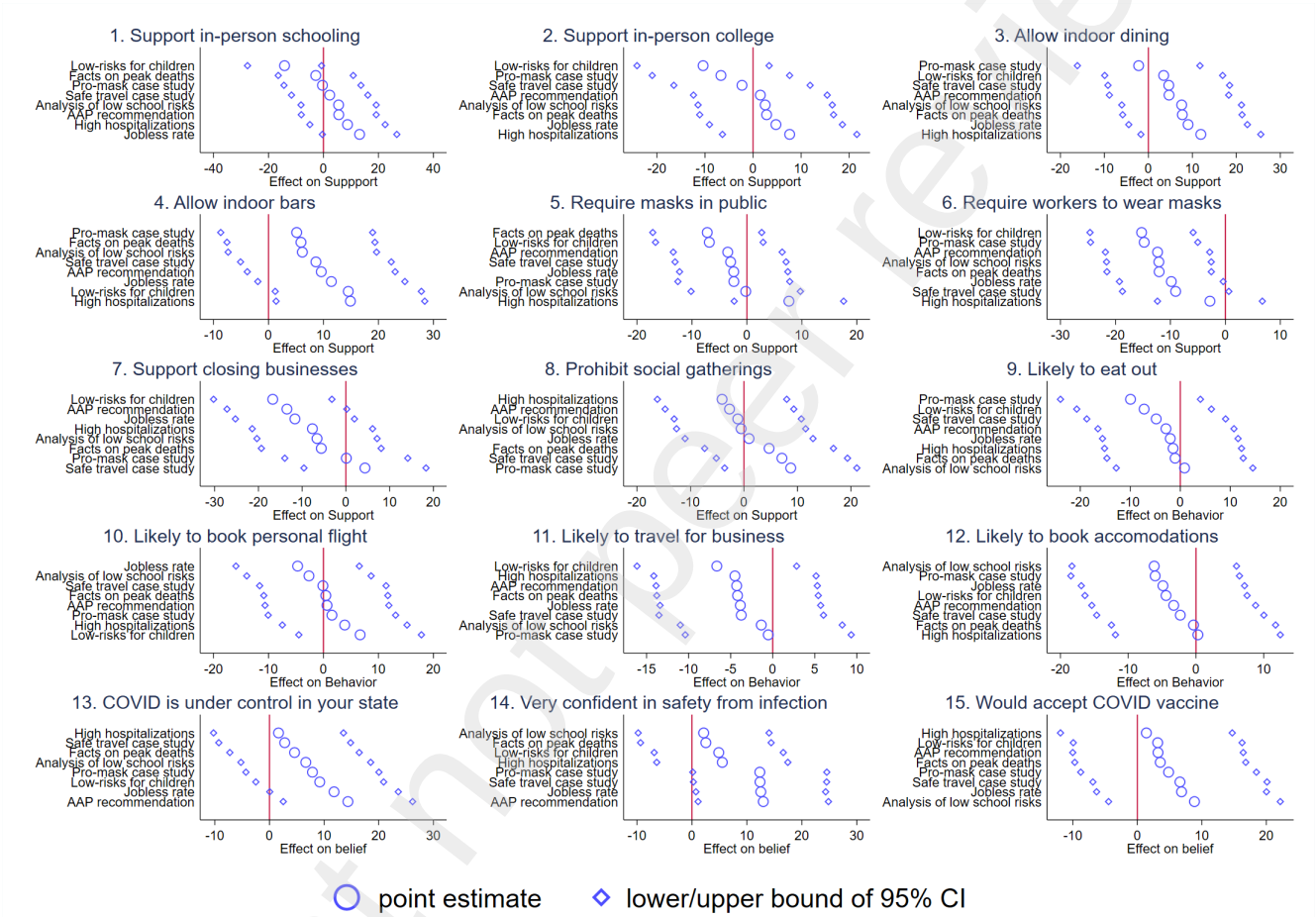
Figure 3: Treatment effects of Democratic Party Members Relative to Republican Party Members



Notes.–Each figure plots treatment effects for respondents identifying as Democrats relative to Republicans. Both Democratic and Republican effects are identified in a model that excludes Independents and is in relation to the experimental reference group. Models 1-8 are policy preferences. Models 9-12 are consumer behaviors, and 13-15 are beliefs. All models control for the demographic characteristics listed in text, including: age, education, race, employment status, political affiliation, numeracy, medical risk factors, household income, health status, and gender. 95% confidence intervals are shown.

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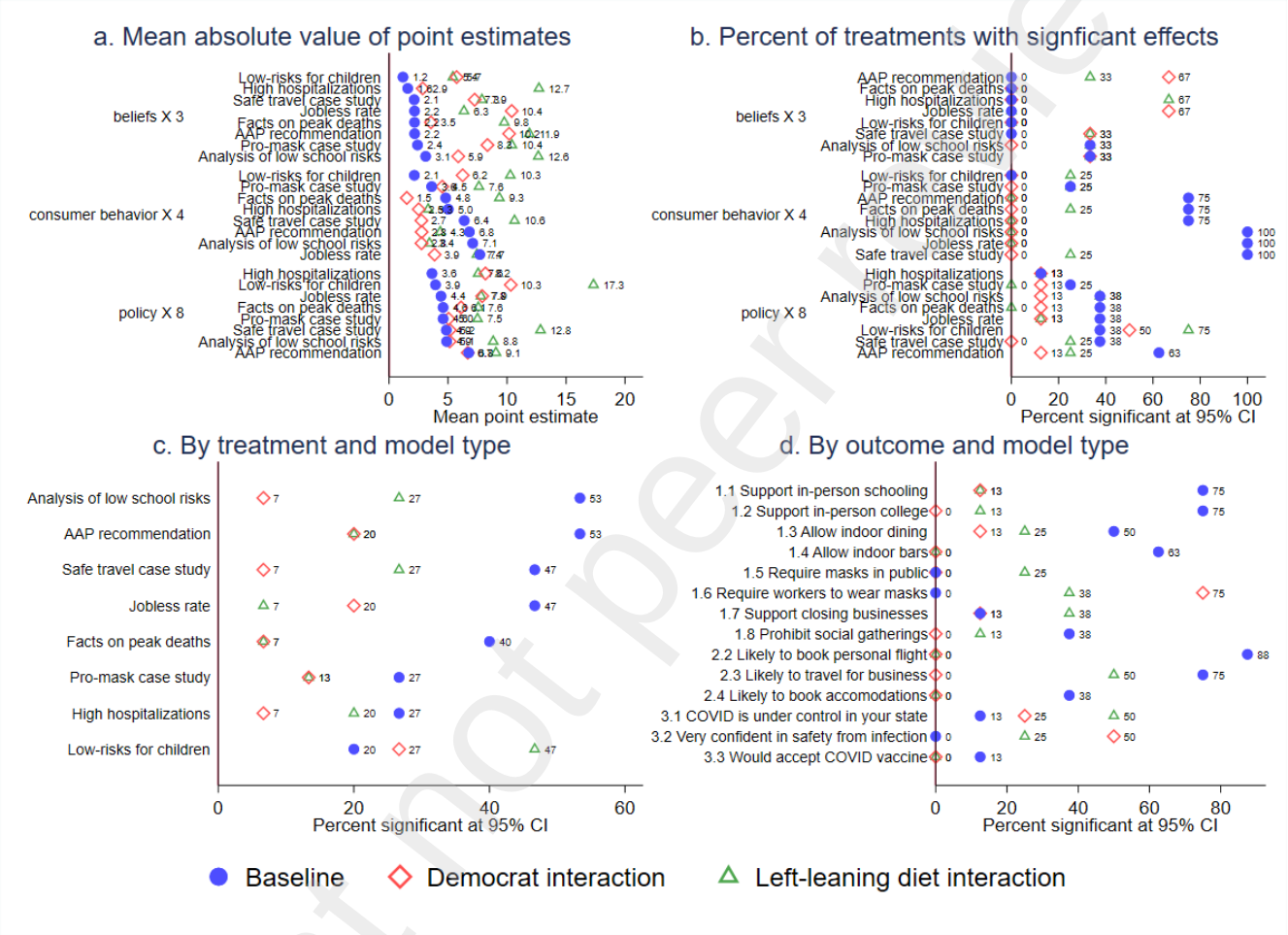
Figure 4: Meta-analysis of treatment effects across model types



Notes.–Each figure plots meta-data from our baseline model with the full set of controls and no interactions, a model that includes an interaction term for Democrats relative to Republicans, and a model that includes an interaction term for consuming a liberal-leaning diet relative to a right-leaning diet. Panel a) shows the mean point estimate for each experimental group organized by the three outcome types. Panel b) show the percentage of treatment effects reaching significance thresholds with 95% confidence intervals for each experimental group organized by the three outcome types. Panel c) shows the percentage of treatment effects reaching significance thresholds with 95% confidence intervals for each experimental group, aggregated across all treatment. Panel d) shows the percentage of treatment effects reaching significance thresholds with 95% confidence intervals for each experimental group, aggregated across all outcomes.

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Supplementary Information for

COVID-19 Information Treatments Shift Related Policy Preferences and Plans for Consumer Behavior

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Sonal Desai

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These supplementary materials start with a discussion of how we classified the partisan orientation of each respondents' news diet. Next, we show findings that did not fit in the main text but may be of interest to readers. The figures show the mean outcomes (policy preferences, consumer intentions, and belief) for each experimental group and for the two major political party across the experimental groups. The regression tables show the full results across all 15 outcomes with point estimates and standard errors for all the variables used in the preferred models.

This PDF file includes:

Discussion: Theory, Methods, and Classification of News Diet

Supplemental Table 1. Mean outcomes by experimental group

Supplemental Table 2. Mean outcomes by experimental group and party

Supplemental Figure 1. Meta analysis of baseline model with all respondents with various assumptions about the optimal set of controls

Supplemental Table 3. Regress experimental assignment on outcomes with demographic controls

Supplemental Table 4. Regress experimental assignment on outcomes with demographic controls and interactions with political party

Supplemental Table 5. Regress experimental assignment on outcomes with demographic controls and interactions with media diet

Supplemental Figure 2. Treatment effects for those with a Left-leaning Diet Relative to those with a Right-leaning Diet

Supplemental Figure 3. Treatment effects in baseline model with no adjustments.

References

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Discussion: Theory, Methods, and Classification of News Diet

Theory: how information heterogeneity can persist and lead to polarized beliefs

There has been a long-held recognition that information as costly to acquire, requiring search and evaluation Stigler (1961). Developing a trusted source for information lowers those costs, but only results in someone being more informed if the trusted source is truly high-quality. We begin with the recognition that the value of information is a function of quality and price. However, unlike other goods, the quality of information is very difficult to know prior to consuming it and is even difficult to know retrospectively. These two facets of information create unique and significant difficulties for consumers. Trust becomes especially important, but trust is disconnected from objective verification, allowing for the formation of stable and heterogeneous beliefs.

Our theory predicts that providing consumers free high-quality information from a trusted source results in the convergence of beliefs. Our presupposition is that consumers generally lack access to low-cost high-quality information. Suppose that the value of information is equal to the quality divided by the price.

We assume that the price of information is increasing with its value because relevant, non-obvious information is costly to discover and disseminate; information is also costly to interpret correctly because the sound interpretation of facts often requires expertise. Traditionally, these costs have been funded by some combination of advertisement fees and subscription fees, and desire for fame or influence persuades many experts or quasi-experts to donate interpretations to the media. Non-media sources of information also face costs, such as government agencies and privately funded surveys, and communication within and across social networks, which are usually unknown to the media and public. The internet and social media platforms, funded via advertisements, have dramatically lowered the price of access to many types of information, including the political views of influential people and those in one's social network.

The price of information is difficult to determine, consisting of partial but vague signals. Platforms that offer news may be free to the consumer but rest on cheap or expensive data gathering processes that are unknown to the consumer. Moreover, consumers do not know, in most cases, how costly it would be for them to acquire the knowledge to answer their questions or discover important information or interpret it correctly.

Information quality is also mostly unknown prior to consumption, and unlike many tradable goods, like automobiles, there are no well-defined specifications. Further, unlike many services, such as movies and restaurants, customer reviews and ratings are not available. The probability that a piece of information will be of the highest quality (e.g. a relevant, insightful, and true fact) has to be estimated and varies from 0 to 1. An individual's degree of knowledge about

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public facts (informativeness) is the aggregate value of all consumed information sources multiplied by the probability that the information has perfect quality.

To summarize, whenever distinct sources of information prevail, informativeness varies because price, quality, and value are all difficult to determine, creating large inefficiencies in the market for information. Many consumers would pay more for higher quality news, but cognitive biases (e.g., party and news source biases) can shape many of the consumption patterns by altering the information that they pay attention to and process.

The news industry attempts to solve these problems by imposing editorial standards and professional codes of conduct, but participants are not forced to adhere to these standards and are often rewarded financially for violating them.

If beliefs are a function of exposure to information, it follows that beliefs will become segmented by information sources and vary based on the quality of those sources. These dynamics create islands of beliefs, with varying correspondence to reality.

Survey details

We launched a novel survey and randomized experiment through a partnership between Gallup and Franklin Templeton, called the Economics of Recovery Study, which took place in six waves from July to December 2020. We use data from wave 6 of the survey, which was fielded between December 1 and December 7th, 2020. Results from this study are based on self-administered web surveys from an opt-in sample provided by Dynata. Gallup weighted the obtained sample to correct for nonresponse.

We make nonresponse adjustments by adjusting the sample to match the national demographics of gender, age, race/ethnicity, region, level of education, marital status, and employment status. Demographic weighting targets are based on the Census Bureau's 2018 data release of the American Community Survey and the Current Population Survey (February 2020).

Our list of control variables included a measure of numeracy, adapted from the Berlin Numeracy Test (Cokely 2012). Our item is based on the following two survey questions:

Please answer the questions below to the best of your ability. Do not use a calculator, but feel free to use paper and pencil.

Q1 Imagine you are throwing one standard (six-sided) die 6 times.

How many times would you expect this die to show the number 1 or the number 6?

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Q2 Imagine that exactly half of people who are infected with a new virus show symptoms. The other half show no symptoms. Now imagine a town of 1,000 people where 200 people are showing symptoms.

How many people in the town would you estimate are infected?

60% of respondents scored “0”, meaning that they got neither right. 26% got one right, and 14% got both right. There was no correlation between this item and political party or news diet, but it was correlated with having a graduate degree (r=.13).

Classification of News Diet

Our survey also collects data on media sources. To avoid making the respondent consider every media source, we start by asking how many hours per week respondents devote to news consumption. 95% responded with a value above zero and were then asked what form of media “do you get most of your news?” Respondents can choose television, radio or podcasts, newspapers, online, or social media. Then, the respondent is presented with a list of 10-15 prominent news sources for whichever form (e.g. *New York Times*, *Wall Street Journal* for print; Fox and CNN for television, etc), and asked to select each one they “normally” consume (i.e. read, watch, or listen to). To code these on a left-right spectrum, we calculate the Pearson correlation coefficient (using survey weights) for an underlying respondent-level indicator of political orientation, using a combination of both their 2020 vote and their stated party affiliation.

Each of the two political variables (vote and party) was coded on a -1,0,1 scale, with 0 indicating neutrality (encompassing non-voters, third-party voters or members, and independents). This political index is correlated with consumption of each media source. The results conform with our subjective impressions of the political orientation of sources and information from other sources. For example, Fox News is strongly oriented towards the right with a value of 0.25; MSNBC and CNN has values of -0.25 and -0.33, respectively; Rush Limbaugh and Sean Hannity radio programs have values of 0.29 and 0.27, respectively; All Things Considered (an NPR program preferred by Democrats) has a value of -0.27. The Wall Street Journal has a value of 0.06, indicating a weak correlation with right-leaning readers, consistent with its centrist orientation. The correlation coefficient is then averaged across individuals to create a mean diet index, which is coded as left, mixed, or right, depending on whether values were <=-0.1, between -0.1 and 0.1, or >=0.1.

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Supplemental Table 1. Mean outcomes by experimental group

	G1. AAP recommendation	G2. Low risks for children	G3. Facts on peak deaths	G4. Record high cases	G5. Analysis of low school risks	G6. Pro-mask case study	G7. Safe travel case study	G8. High hospitalizations	G9. Jobless rate
Percent who support policy									
Providing in-person schooling for K-12 students	66.2%	61.5%	56.4%	50.5%	64.4%	60.6%	60.1%	56.1%	61.4%
Providing in-person schooling for college students	60.3%	57.8%	55.6%	47.6%	56.7%	55.0%	56.5%	51.6%	56.9%
Allowing indoor dining at restaurants	51.2%	49.4%	50.3%	43.8%	47.7%	51.0%	50.3%	45.8%	51.4%
Allowing indoor seating at bars	44.0%	39.4%	44.6%	34.8%	40.9%	41.0%	41.5%	39.7%	39.3%
Requiring people to wear masks in indoor public spaces	84.4%	85.7%	84.8%	86.1%	84.2%	82.3%	84.4%	85.7%	84.8%
Requiring workers to wear mask	85.2%	87.1%	86.7%	84.7%	85.5%	85.5%	86.9%	87.7%	86.1%
Requiring non-essential businesses to close in-person operations	47.9%	54.2%	51.8%	55.1%	51.5%	48.4%	51.4%	57.1%	51.7%
Prohibiting large indoor social gatherings	74.5%	78.5%	73.5%	72.3%	74.6%	74.1%	75.2%	78.7%	73.6%
Consumer behavior: percent likely (4 or 3 on 1-4 scale)									
Eat at a restaurant	51.0%	44.3%	46.7%	39.5%	50.5%	42.9%	45.5%	43.8%	47.0%
Book a flight for personal travel	25.2%	19.8%	23.0%	18.5%	25.3%	23.4%	24.9%	21.6%	24.3%
Book a flight for business travel	18.4%	14.9%	17.8%	13.6%	16.2%	15.9%	17.4%	16.4%	19.0%
Book a hotel room or vacation rental	28.2%	23.4%	28.4%	24.1%	27.4%	25.3%	28.9%	24.4%	30.4%
Beliefs									
Believe the coronavirus is under control in your state	26.7%	25.8%	24.5%	25.8%	20.6%	25.0%	24.7%	22.1%	26.3%
Confidence in safety from COVID (4 on 1-4 scale)	24.0%	20.8%	22.9%	23.1%	20.7%	20.9%	18.3%	23.4%	24.3%
Agree to be vaccinated in Spring 2021 with COVID vaccine	51.8%	53.2%	52.5%	55.0%	50.4%	49.1%	51.4%	51.3%	55.5%

Note: Table reports means using sample weight for each of the nine experimental groups. Group 4 is used as reference in analysis. Sample size is approximately 554 to 557 observations per experimental group. Standard errors are approximately 2% for each cell.

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Supplemental Table 2. Mean outcomes by experimental group for Democrats and Republicans

	G1. AAP recommendation	G2. Low risks for children	G3. Facts on peak deaths	G4. Record high cases	G5. Analysis of low school risks	G6. Pro-mask case study	G7. Safe travel case study	G8. High hospitalizations	G9. Jobless rate
Democrats									
Percent who support policy									
Providing in-person schooling for K-12 students	62.1%	49.7%	50.8%	45.0%	59.3%	52.2%	56.9%	54.3%	59.7%
Providing in-person schooling for college students	53.5%	46.2%	51.4%	40.4%	51.4%	44.0%	51.7%	48.3%	51.0%
Allowing indoor dining at restaurants	41.8%	36.9%	40.0%	28.8%	34.5%	33.1%	37.2%	36.5%	39.4%
Allowing indoor seating at bars	36.1%	30.6%	34.3%	21.3%	25.2%	26.9%	33.0%	31.4%	29.6%
Requiring people to wear masks in indoor public spaces	90.2%	91.0%	88.3%	91.9%	94.4%	88.3%	89.1%	92.1%	89.2%
Requiring workers to wear mask	90.1%	92.4%	92.1%	92.6%	89.8%	89.1%	91.1%	94.8%	90.9%
Requiring non-essential businesses to close in-person operations	59.0%	62.2%	66.3%	67.6%	67.8%	65.9%	67.2%	66.2%	64.1%
Prohibiting large indoor social gatherings	81.1%	87.7%	85.1%	81.1%	83.6%	86.7%	85.3%	83.9%	82.9%
Consumer behavior: percent likely (4 or 3 on 1-4 scale)									
Eat at a restaurant	43.0%	30.4%	38.4%	32.3%	44.0%	28.1%	37.5%	37.2%	36.6%
Book a flight for personal travel	25.7%	19.2%	22.5%	17.7%	22.4%	20.3%	22.6%	22.0%	19.3%
Book a flight for business travel	20.9%	13.4%	17.9%	14.2%	14.4%	15.6%	19.2%	15.3%	18.5%
Book a hotel room or vacation rental	26.5%	19.0%	26.9%	22.5%	23.0%	20.3%	26.2%	25.2%	27.7%
Beliefs									
Believe the coronavirus is under control in your state	25.1%	21.6%	16.8%	19.4%	12.8%	19.6%	19.1%	14.0%	23.9%
Confidence in safety from COVID (4 on 1-4 scale)	25.6%	17.6%	17.8%	19.5%	17.2%	22.4%	17.6%	21.0%	25.5%
Agree to be vaccinated in Spring 2021 with COVID vaccine	61.6%	65.2%	64.5%	63.4%	60.8%	63.5%	63.3%	61.0%	66.5%
Republicans									
Percent who support policy									
Providing in-person schooling for K-12 students	76.4%	81.3%	73.2%	62.9%	72.2%	73.1%	74.0%	65.3%	66.5%
Providing in-person schooling for college students	72.4%	75.5%	69.1%	60.8%	69.0%	69.7%	71.9%	61.0%	66.2%
Allowing indoor dining at restaurants	73.3%	69.7%	72.5%	66.8%	64.1%	74.8%	68.1%	59.8%	68.2%
Allowing indoor seating at bars	59.9%	52.7%	66.1%	57.1%	57.6%	57.8%	57.9%	51.2%	52.7%
Requiring people to wear masks in indoor public spaces	82.6%	83.9%	80.2%	76.7%	78.5%	78.1%	80.4%	75.0%	79.4%
Requiring workers to wear mask	82.1%	86.1%	82.6%	72.9%	80.6%	81.6%	81.3%	80.2%	81.2%
Requiring non-essential businesses to close in-person operations	38.2%	44.9%	39.2%	36.6%	38.4%	29.0%	31.4%	41.5%	40.5%
Prohibiting large indoor social gatherings	66.0%	70.7%	57.9%	61.7%	66.2%	60.7%	61.6%	71.1%	63.7%
Consumer behavior: percent likely (4 or 3 on 1-4 scale)									
Eat at a restaurant	66.2%	60.8%	62.9%	56.3%	67.2%	62.9%	61.2%	58.3%	60.1%
Book a flight for personal travel	29.9%	18.4%	27.7%	27.3%	32.8%	27.6%	28.5%	25.3%	28.0%
Book a flight for business travel	21.9%	17.6%	20.4%	17.7%	17.4%	15.2%	20.1%	16.5%	17.4%
Book a hotel room or vacation rental	35.1%	27.5%	32.7%	32.6%	36.7%	33.2%	33.0%	27.2%	35.3%
Beliefs									
Believe the coronavirus is under control in your state	31.2%	35.0%	36.3%	40.9%	29.6%	35.1%	37.1%	33.2%	29.9%
Confidence in safety from COVID (4 on 1-4 scale)	28.0%	26.6%	32.8%	32.2%	29.9%	26.9%	20.0%	28.0%	26.4%

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Agree to be vaccinated in Spring 2021 with COVID vaccine	51.2%	47.6%	47.3%	50.2%	40.7%	45.3%	48.8%	48.9%	49.9%

Note: Table reports means using sample weight for each of the nine experimental groups. Group 4 is used as reference in analysis. Sample size is approximately 154 to 188 for Republican and 202 to 237 for Democrats for each experimental group. Standard errors range from approximately 2.0% to 3.8% for each cell. Details are available upon request.

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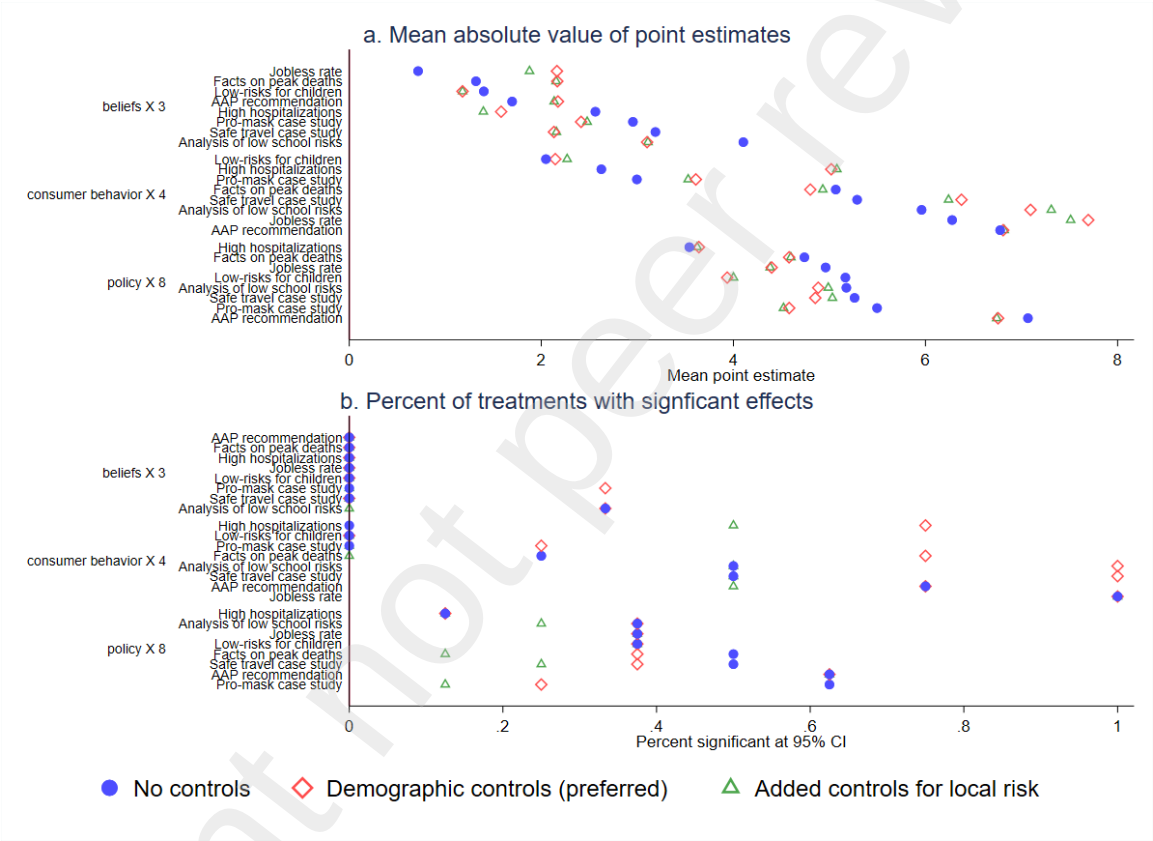
Supplemental Figure 1. Meta analysis of baseline model with all respondents with various assumptions about the optimal set of controls. Top panel shows the mean absolute point estimates of experimental effects on outcomes from a model with no control variables (solid blue dot), our preferred model with demographic controls for all adults (red triangle), and an additional model with all of the demographic controls plus controls for local COVID-19 risk (the log of cumulative COVID-19 deaths per capita in the respondent's county through November 2020 from USA Facts and county population density from the Census Bureau and American Community Survey). The bottom panel shows the percentage of coefficients from these models that reach significance at 95% confidence intervals. Outcomes are categorized as beliefs, consumer behavior, and policy.



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Supplemental Table 3. Regress experimental condition on all outcomes, controlling for full set of demographic variables

	(1) Support in-person schools	(2) Support In-person school college	(3) Support Indoor dining at restaurants	(4) Support Indoor seating at bars	(5) Support requiring masks in indoor public spaces	(6) Support requiring that workers wear masks with others	(7) Support closing non-essential business temp close in-person	(8) Support prohibiting large indoor gatherings	(9) Behavior-Likely to eat out	(10) Behavior-Likely to book personal flight	(11) Behavior-Likely to book business flight	(12) Behavior-Likely to book hotel	(13) Believe COVID is under control in your state	(14) Very confident can protect oneself	(15) Agree to effective COVID vaccine in Spring
Experiment-Show AAP recommendation	0.154***	0.119***	0.068**	0.071**	-0.020	-0.003	-0.085***	0.020	0.116***	0.065***	0.043**	0.049*	0.003	0.012	-0.050*
	(0.030)	(0.030)	(0.030)	(0.029)	(0.021)	(0.021)	(0.030)	(0.026)	(0.029)	(0.024)	(0.020)	(0.025)	(0.025)	(0.025)	(0.028)
Experiment-Explain low deaths among children	0.089***	0.076**	0.028	0.015	-0.010	0.022	-0.009	0.065**	0.048*	0.015	0.020	-0.003	0.007	-0.018	-0.010
	(0.030)	(0.030)	(0.030)	(0.029)	(0.022)	(0.021)	(0.030)	(0.026)	(0.029)	(0.024)	(0.020)	(0.026)	(0.025)	(0.025)	(0.028)
Experiment-Explain trend in deaths	0.049*	0.067**	0.065**	0.078***	-0.012	0.036*	-0.027	0.034	0.064**	0.049**	0.040**	0.039	-0.030	-0.015	-0.020
	(0.030)	(0.030)	(0.030)	(0.029)	(0.022)	(0.021)	(0.030)	(0.026)	(0.029)	(0.024)	(0.020)	(0.026)	(0.025)	(0.025)	(0.028)
Experiment-Schools are safe	0.133***	0.085***	0.031	0.041	-0.006	0.021	-0.021	0.054**	0.107***	0.084***	0.042**	0.051**	-0.057**	-0.021	-0.016
	(0.030)	(0.031)	(0.030)	(0.030)	(0.022)	(0.021)	(0.030)	(0.026)	(0.030)	(0.024)	(0.021)	(0.026)	(0.026)	(0.025)	(0.029)
Experiment-Case showing masks prevent infections at salon	0.083***	0.059*	0.063**	0.050*	-0.031	0.012	-0.036	0.032	0.036	0.051**	0.040*	0.017	0.002	-0.013	-0.058**
	(0.030)	(0.031)	(0.030)	(0.030)	(0.022)	(0.021)	(0.030)	(0.026)	(0.030)	(0.024)	(0.021)	(0.026)	(0.026)	(0.025)	(0.029)
Experiment-Case showing flights are safe	0.086***	0.073**	0.059**	0.039	-0.019	0.029	-0.034	0.049*	0.066**	0.081***	0.043**	0.065**	-0.012	-0.042*	-0.010
	(0.030)	(0.030)	(0.030)	(0.029)	(0.022)	(0.021)	(0.030)	(0.026)	(0.029)	(0.024)	(0.021)	(0.026)	(0.025)	(0.025)	(0.029)
Experiment-Hospitalizations are rising	0.050*	0.033	0.024	0.041	-0.008	0.024	0.025	0.086***	0.061**	0.061**	0.044**	0.035	-0.032	0.007	-0.008
	(0.029)	(0.030)	(0.029)	(0.029)	(0.021)	(0.020)	(0.029)	(0.026)	(0.029)	(0.024)	(0.020)	(0.025)	(0.025)	(0.025)	(0.028)
Experiment-Current unemployment rate	0.096***	0.075**	0.078***	0.028	-0.009	0.021	-0.024	0.021	0.088***	0.074***	0.067***	0.080***	0.013	0.034	0.018
	(0.030)	(0.030)	(0.030)	(0.029)	(0.022)	(0.021)	(0.030)	(0.026)	(0.029)	(0.024)	(0.020)	(0.026)	(0.025)	(0.025)	(0.028)
Annual flights for work in 2019	0.005***	0.004**	0.005***	0.006***	-0.002**	0.001	0.005***	-0.001	0.004**	0.007***	0.010***	0.005***	0.009***	0.003**	0.004***
	(0.002)	(0.002)	(0.002)	(0.002)	(0.001)	(0.001)	(0.002)	(0.001)	(0.002)	(0.001)	(0.001)	(0.001)	(0.001)	(0.001)	(0.002)
Annual flights for pleasure in 2019	0.002	-0.000	0.003	0.003	-0.000	-0.005***	-0.000	-0.001	0.006***	0.012***	0.004***	0.009***	0.005***	0.002	0.003
	(0.002)	(0.002)	(0.002)	(0.002)	(0.001)	(0.001)	(0.002)	(0.002)	(0.002)	(0.002)	(0.001)	(0.002)	(0.002)	(0.002)	(0.002)

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Numeracy--number correct on 3-item test	-0.043***	-0.044***	-0.032***	-0.037***	0.028***	0.025***	-0.015	0.023**	-0.045***	-0.051***	-0.070***	-0.059***	-0.054***	-0.049***	0.039***
	(0.010)	(0.010)	(0.010)	(0.010)	(0.007)	(0.007)	(0.010)	(0.009)	(0.010)	(0.008)	(0.007)	(0.009)	(0.009)	(0.009)	(0.010)
Currently invested in stock market	0.053***	0.020	0.025	0.021	0.047***	0.047***	0.007	0.047***	0.039**	0.056***	0.060***	0.078***	0.058***	0.001	0.090***
	(0.017)	(0.017)	(0.017)	(0.016)	(0.012)	(0.012)	(0.017)	(0.015)	(0.016)	(0.013)	(0.011)	(0.014)	(0.014)	(0.014)	(0.016)
Employed	0.036**	0.036**	0.043***	0.067***	-0.015	-0.023*	-0.025	-0.045***	0.082***	0.057***	0.066***	0.061***	0.028**	0.021	-0.006
	(0.017)	(0.017)	(0.017)	(0.016)	(0.012)	(0.012)	(0.017)	(0.015)	(0.016)	(0.013)	(0.011)	(0.014)	(0.014)	(0.014)	(0.016)
Homeowner	0.005	0.008	0.006	0.000	-0.008	-0.018	-0.017	-0.015	0.030*	0.029**	0.026**	0.016	0.014	0.016	0.026
	(0.017)	(0.017)	(0.017)	(0.017)	(0.012)	(0.012)	(0.017)	(0.015)	(0.017)	(0.014)	(0.012)	(0.015)	(0.014)	(0.014)	(0.016)
Household income (1-10 categories)	0.006	0.008*	-0.005	-0.006	-0.000	0.002	-0.006	0.011***	-0.004	-0.002	-0.003	-0.003	-0.008**	-0.010***	-0.002
	(0.004)	(0.004)	(0.004)	(0.004)	(0.003)	(0.003)	(0.004)	(0.003)	(0.004)	(0.003)	(0.003)	(0.003)	(0.003)	(0.003)	(0.004)
Pre-COVID household budget-Indebtedness (5) vs savings (1)	-0.023***	-0.018**	-0.027***	-0.016**	-0.019***	-0.015***	-0.003	-0.002	-0.022***	-0.027***	-0.025***	-0.026***	-0.031***	-0.062***	-0.040***
	(0.008)	(0.008)	(0.008)	(0.008)	(0.006)	(0.006)	(0.008)	(0.007)	(0.008)	(0.007)	(0.006)	(0.007)	(0.007)	(0.007)	(0.008)
Democrat	-0.020	-0.037**	-0.104***	-0.084***	0.057***	0.053***	0.133***	0.086***	-0.062***	-0.002	-0.002	-0.006	-0.036**	0.002	0.182***
	(0.017)	(0.018)	(0.017)	(0.017)	(0.013)	(0.012)	(0.017)	(0.015)	(0.017)	(0.014)	(0.012)	(0.015)	(0.015)	(0.015)	(0.017)
Republican	0.149***	0.164***	0.228***	0.199***	-0.050***	-0.057***	-0.099***	-0.110***	0.212***	0.083***	0.045***	0.106***	0.139***	0.098***	-0.020
	(0.018)	(0.019)	(0.018)	(0.018)	(0.013)	(0.013)	(0.018)	(0.016)	(0.018)	(0.015)	(0.013)	(0.016)	(0.016)	(0.016)	(0.018)
Less than high school	-0.048	-0.014	-0.036	0.027	-0.021	-0.007	0.091**	0.054	0.044	0.039	0.075***	0.060*	0.019	-0.064*	-0.074*
	(0.042)	(0.043)	(0.042)	(0.041)	(0.030)	(0.029)	(0.042)	(0.037)	(0.041)	(0.034)	(0.029)	(0.036)	(0.036)	(0.035)	(0.040)
Some college without degree	-0.026	-0.033	0.014	0.043**	-0.007	-0.008	-0.011	-0.005	0.017	0.029*	0.002	0.002	-0.001	-0.007	0.034*
	(0.021)	(0.022)	(0.021)	(0.021)	(0.015)	(0.015)	(0.021)	(0.019)	(0.021)	(0.017)	(0.015)	(0.018)	(0.018)	(0.018)	(0.020)
Industry certification or certificate	0.021	0.029	0.057	0.087**	-0.053*	-0.041	-0.016	0.041	0.100***	0.038	0.052**	0.054*	0.003	-0.020	0.005
	(0.037)	(0.038)	(0.037)	(0.037)	(0.027)	(0.026)	(0.037)	(0.033)	(0.037)	(0.030)	(0.026)	(0.032)	(0.032)	(0.032)	(0.036)
Associates degree	-0.023	-0.041	0.018	0.014	-0.032*	-0.024	-0.041	0.008	-0.012	0.022	-0.014	0.019	-0.012	-0.054**	0.030
	(0.026)	(0.026)	(0.026)	(0.025)	(0.019)	(0.018)	(0.026)	(0.022)	(0.025)	(0.021)	(0.018)	(0.022)	(0.022)	(0.022)	(0.024)
Bachelor's degree	-0.006	-0.026	-0.033	-0.026	0.008	0.010	0.010	0.016	-0.019	0.024	0.003	0.003	-0.011	-0.030	0.099***
	(0.022)	(0.022)	(0.022)	(0.021)	(0.016)	(0.015)	(0.022)	(0.019)	(0.021)	(0.018)	(0.015)	(0.019)	(0.018)	(0.018)	(0.021)
Graduate degree	-0.017	-0.040	-0.042	-0.035	0.006	-0.000	0.065**	0.038	-0.048*	0.061***	0.032*	0.003	0.006	-0.007	0.127***

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	(0.027)	(0.028)	(0.027)	(0.027)	(0.020)	(0.019)	(0.027)	(0.024)	(0.027)	(0.022)	(0.019)	(0.023)	(0.023)	(0.023)	(0.026)
Black	-0.046*	-0.029	-0.064***	-0.045*	0.041**	0.045***	0.087***	-0.017	-0.022	0.024	0.027*	0.076***	-0.016	0.110***	-0.149***
	(0.024)	(0.024)	(0.024)	(0.023)	(0.017)	(0.017)	(0.024)	(0.021)	(0.024)	(0.019)	(0.016)	(0.021)	(0.020)	(0.020)	(0.023)
Hispanic	0.036*	0.015	-0.053**	-0.029	0.005	-0.011	0.007	-0.006	-0.023	0.044***	0.054***	0.058***	0.072***	-0.013	0.005
	(0.021)	(0.022)	(0.021)	(0.021)	(0.015)	(0.015)	(0.021)	(0.019)	(0.021)	(0.017)	(0.015)	(0.018)	(0.018)	(0.018)	(0.020)
Asian	-0.043	-0.040	-0.092***	-0.112***	0.013	0.046**	0.177***	0.054**	-0.128***	-0.010	-0.032	-0.021	0.020	-0.072***	0.005
	(0.031)	(0.032)	(0.031)	(0.030)	(0.022)	(0.021)	(0.031)	(0.027)	(0.031)	(0.025)	(0.021)	(0.027)	(0.026)	(0.026)	(0.030)
Other race	0.041	0.047	0.018	-0.038	-0.014	-0.007	0.021	-0.045	0.029	0.093**	-0.001	0.006	0.019	0.148***	-0.140***
	(0.048)	(0.049)	(0.048)	(0.048)	(0.035)	(0.034)	(0.048)	(0.042)	(0.048)	(0.039)	(0.033)	(0.042)	(0.042)	(0.041)	(0.046)
Age 18-29	-0.020	0.009	0.076***	0.140***	-0.085***	-0.134***	0.156***	-0.059**	0.209***	0.233***	0.271***	0.218***	0.200***	0.075***	-0.090***
	(0.027)	(0.027)	(0.027)	(0.026)	(0.019)	(0.019)	(0.027)	(0.023)	(0.026)	(0.022)	(0.018)	(0.023)	(0.023)	(0.022)	(0.026)
Age 30-49	-0.061***	-0.016	0.016	0.098***	-0.065***	-0.086***	0.148***	-0.036*	0.091***	0.169***	0.195***	0.184***	0.131***	0.108***	-0.160***
	(0.023)	(0.023)	(0.023)	(0.022)	(0.017)	(0.016)	(0.023)	(0.020)	(0.023)	(0.019)	(0.016)	(0.020)	(0.019)	(0.019)	(0.022)
Age 50-64	-0.065***	-0.053**	-0.032	0.009	-0.043***	-0.045***	0.042*	-0.045**	-0.002	0.061***	0.040***	0.046**	0.030	0.022	-0.138***
	(0.022)	(0.022)	(0.022)	(0.021)	(0.016)	(0.015)	(0.022)	(0.019)	(0.022)	(0.018)	(0.015)	(0.019)	(0.019)	(0.018)	(0.021)
Male	0.044***	0.029*	0.041***	0.051***	-0.034***	-0.050***	-0.011	-0.050***	0.053***	0.068***	0.079***	0.085***	0.051***	0.048***	0.142***
	(0.015)	(0.015)	(0.015)	(0.015)	(0.011)	(0.010)	(0.015)	(0.013)	(0.015)	(0.012)	(0.010)	(0.013)	(0.013)	(0.013)	(0.014)
I have a high-risk medical condition	0.059***	0.028	0.016	0.004	0.035***	-0.009	0.119***	0.065***	0.019	0.039***	0.071***	0.051***	0.041***	0.016	0.112***
	(0.017)	(0.018)	(0.017)	(0.017)	(0.013)	(0.012)	(0.017)	(0.015)	(0.017)	(0.014)	(0.012)	(0.015)	(0.015)	(0.015)	(0.017)
Family member has a high-risk medical condition	0.053***	0.025	-0.009	-0.029	0.055***	0.027*	0.073***	0.051***	-0.033*	-0.011	0.012	0.007	-0.010	-0.037**	0.063***
	(0.020)	(0.020)	(0.020)	(0.019)	(0.014)	(0.014)	(0.020)	(0.017)	(0.020)	(0.016)	(0.014)	(0.017)	(0.017)	(0.017)	(0.019)
I and family member have a high-risk medical condition	-0.032	-0.081***	-0.071***	-0.069***	0.067***	0.051***	0.146***	0.073***	-0.056**	-0.026	0.041**	-0.020	-0.011	-0.058***	0.101***
	(0.026)	(0.026)	(0.026)	(0.025)	(0.019)	(0.018)	(0.026)	(0.022)	(0.025)	(0.021)	(0.018)	(0.022)	(0.022)	(0.022)	(0.025)
Constant	0.481***	0.464***	0.465***	0.293***	0.923***	0.946***	0.410***	0.690***	0.266***	-0.054	-0.089***	0.001	0.157***	0.311***	0.469***
	(0.042)	(0.043)	(0.042)	(0.042)	(0.031)	(0.029)	(0.042)	(0.037)	(0.042)	(0.034)	(0.029)	(0.037)	(0.036)	(0.036)	(0.041)
Observations	4,593	4,593	4,593	4,593	4,593	4,593	4,593	4,593	4,595	4,595	4,595	4,595	4,580	4,587	4,587

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Adjusted R-squared	0.066	0.054	0.106	0.101	0.042	0.056	0.104	0.060	0.118	0.156	0.214	0.139	0.119	0.076	0.177

Robust standard errors in parentheses, *** p<0.01, ** p<0.05, * p<0.1

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Supplemental Table 4. Regress experimental condition on all outcomes with party interactions, controlling for full set of demographic variables. Sample restricted to Republicans and Democrats

	(1) Support in-person schools	(2) Support In-person school college	(3) Support Indoor dining at restaurants	(4) Support Indoor seating at bars	(5) Support requiring masks in indoor public spaces	(6) Support requiring that workers wear masks with others	(7) Support closing non-essential business temp close in-person	(8) Support prohibiting large indoor gatherings	(9) Behavior-Likely to eat out	(10) Behavior-Likely to book personal flight	(11) Behavior-Likely to book business flight	(12) Behavior-Likely to book hotel	(13) Believe COVID is under control in your state	(14) Very confident can protect oneself	(15) Agree to effective COVID vaccine in Spring
Experiment-Show AAP recommendation	0.118**	0.094*	0.055	0.005	0.040	0.091**	0.040	0.041	0.116**	0.055	0.070*	0.062	-0.111**	-0.069	-0.044
	(0.053)	(0.054)	(0.052)	(0.052)	(0.038)	(0.037)	(0.053)	(0.047)	(0.053)	(0.044)	(0.037)	(0.047)	(0.046)	(0.046)	(0.051)
Experiment-Explain low deaths among children	0.167***	0.125**	0.025	-0.068	0.058	0.139***	0.123**	0.085*	0.071	-0.041	0.052	0.005	-0.048	-0.058	-0.007
	(0.052)	(0.053)	(0.051)	(0.051)	(0.037)	(0.036)	(0.052)	(0.046)	(0.052)	(0.043)	(0.036)	(0.046)	(0.045)	(0.045)	(0.050)
Experiment-Explain trend in deaths	0.083	0.061	0.030	0.047	0.037	0.117***	0.050	0.004	0.066	0.040	0.057	0.031	-0.063	-0.040	-0.027
	(0.053)	(0.054)	(0.053)	(0.052)	(0.038)	(0.037)	(0.053)	(0.047)	(0.053)	(0.044)	(0.037)	(0.047)	(0.046)	(0.046)	(0.051)
Experiment-Schools are safe	0.092*	0.078	-0.014	-0.018	0.029	0.095***	0.065	0.066	0.133***	0.100**	0.040	0.098**	-0.113**	-0.034	-0.083*
	(0.051)	(0.052)	(0.051)	(0.050)	(0.037)	(0.036)	(0.051)	(0.045)	(0.051)	(0.042)	(0.036)	(0.045)	(0.044)	(0.044)	(0.050)
Experiment-Case showing masks prevent infections at salon	0.061	0.063	0.051	-0.017	-0.005	0.093**	-0.011	-0.024	0.064	0.014	0.016	0.041	-0.057	-0.076	-0.075
	(0.053)	(0.054)	(0.053)	(0.053)	(0.039)	(0.037)	(0.054)	(0.047)	(0.053)	(0.044)	(0.038)	(0.047)	(0.046)	(0.046)	(0.052)
Experiment-Case showing flights are safe	0.098*	0.107**	0.018	-0.017	0.014	0.085**	-0.037	-0.002	0.077	0.049	0.061	0.048	-0.032	-0.140***	-0.014
	(0.054)	(0.055)	(0.053)	(0.053)	(0.039)	(0.037)	(0.054)	(0.047)	(0.054)	(0.044)	(0.038)	(0.048)	(0.046)	(0.047)	(0.052)
Experiment-Hospitalizations are rising	0.005	-0.009	-0.032	-0.051	-0.058	0.051	0.068	0.091**	0.098*	0.047	0.050	0.031	-0.059	-0.042	-0.002
	(0.052)	(0.053)	(0.051)	(0.051)	(0.038)	(0.036)	(0.052)	(0.046)	(0.052)	(0.043)	(0.036)	(0.046)	(0.045)	(0.045)	(0.050)
Experiment-Current unemployment rate	0.021	0.045	0.023	-0.050	0.013	0.085**	0.075	0.019	0.078	0.067	0.072**	0.101**	-0.073	-0.035	-0.008
	(0.052)	(0.053)	(0.051)	(0.051)	(0.037)	(0.036)	(0.052)	(0.045)	(0.052)	(0.043)	(0.036)	(0.046)	(0.045)	(0.045)	(0.050)
Annual flights for work in 2019	0.004**	0.003*	0.006***	0.006***	-0.003**	0.001	0.005***	-0.001	0.005***	0.008***	0.011***	0.007***	0.009***	0.004**	0.006***
	(0.002)	(0.002)	(0.002)	(0.002)	(0.001)	(0.001)	(0.002)	(0.002)	(0.002)	(0.001)	(0.001)	(0.002)	(0.002)	(0.002)	(0.002)
Annual flights for pleasure in 2019	0.002	-0.001	0.000	0.003	0.002	-0.002	0.000	0.000	0.004*	0.010***	0.002	0.006***	0.004**	0.001	0.002
	(0.002)	(0.002)	(0.002)	(0.002)	(0.002)	(0.002)	(0.002)	(0.002)	(0.002)	(0.002)	(0.002)	(0.002)	(0.002)	(0.002)	(0.002)
Numeracy--number correct on 3-item test	-0.051***	-0.059***	-0.041***	-0.041***	0.018**	0.015*	-0.014	0.015	-0.043***	-0.053***	-0.074***	-0.052***	-0.058***	-0.050***	0.039***
	(0.012)	(0.012)	(0.012)	(0.012)	(0.009)	(0.008)	(0.012)	(0.010)	(0.012)	(0.010)	(0.008)	(0.010)	(0.010)	(0.010)	(0.011)
Currently invested in stock market	0.049**	0.037*	0.034*	0.041**	0.038***	0.041***	-0.021	0.038**	0.051***	0.070***	0.083***	0.103***	0.068***	0.017	0.077***

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	(0.019)	(0.020)	(0.019)	(0.019)	(0.014)	(0.013)	(0.019)	(0.017)	(0.019)	(0.016)	(0.014)	(0.017)	(0.017)	(0.017)	(0.019)
Employed	0.053***	0.068***	0.056***	0.077***	-0.011	-0.017	-0.029	-0.036**	0.073***	0.058***	0.060***	0.062***	0.028	0.016	-0.011
	(0.020)	(0.020)	(0.020)	(0.020)	(0.014)	(0.014)	(0.020)	(0.018)	(0.020)	(0.016)	(0.014)	(0.018)	(0.017)	(0.017)	(0.019)
Homeowner	-0.010	-0.014	0.012	-0.018	-0.000	-0.018	-0.017	-0.019	0.014	0.017	0.013	-0.011	0.011	0.015	0.021
	(0.020)	(0.020)	(0.020)	(0.020)	(0.014)	(0.014)	(0.020)	(0.018)	(0.020)	(0.016)	(0.014)	(0.018)	(0.017)	(0.017)	(0.019)
Household income (1-10 categories)	0.007	0.011**	-0.002	-0.003	0.001	0.004	-0.001	0.013***	-0.002	0.002	0.001	-0.001	-0.007*	-0.006	0.001
	(0.005)	(0.005)	(0.005)	(0.005)	(0.003)	(0.003)	(0.005)	(0.004)	(0.005)	(0.004)	(0.003)	(0.004)	(0.004)	(0.004)	(0.004)
Pre-COVID household budget-Indebtedness (5) vs savings (1)	-0.022***	-0.022**	-0.024**	-0.014	-0.010	-0.013*	0.001	0.002	-0.019***	-0.027***	-0.021***	-0.027***	-0.033***	-0.063***	-0.048***
	(0.010)	(0.010)	(0.010)	(0.009)	(0.007)	(0.007)	(0.010)	(0.008)	(0.010)	(0.008)	(0.007)	(0.009)	(0.008)	(0.008)	(0.009)
Democrat									-0.249***	-0.096**	-0.015	-0.087**	-0.249***	-0.170***	-
									(0.050)	(0.041)	(0.035)	(0.044)	(0.043)	(0.043)	
Republican	0.200***	0.210***	0.393***	0.380***	-0.131***	-0.214***	-0.303***	-0.194***							-0.161***
	(0.050)	(0.051)	(0.050)	(0.049)	(0.036)	(0.035)	(0.050)	(0.044)							(0.048)
Less than high school	0.023	0.125**	0.027	0.055	0.052	0.015	0.169***	0.063	0.035	0.076	0.082**	0.121**	0.007	-0.058	-0.104*
	(0.056)	(0.057)	(0.056)	(0.055)	(0.041)	(0.039)	(0.056)	(0.050)	(0.056)	(0.046)	(0.040)	(0.050)	(0.049)	(0.049)	(0.055)
Some college without degree	-0.024	-0.044*	-0.009	0.024	0.003	-0.002	0.006	0.005	0.002	0.031	0.011	0.002	-0.020	-0.013	0.017
	(0.024)	(0.025)	(0.024)	(0.024)	(0.018)	(0.017)	(0.025)	(0.022)	(0.024)	(0.020)	(0.017)	(0.022)	(0.021)	(0.021)	(0.024)
Industry certification or certificate	0.007	-0.021	0.036	0.098**	-0.059*	-0.065**	-0.009	0.071*	0.069	0.018	0.044	0.031	-0.017	0.007	0.002
	(0.043)	(0.044)	(0.043)	(0.042)	(0.031)	(0.030)	(0.043)	(0.038)	(0.043)	(0.036)	(0.030)	(0.038)	(0.037)	(0.038)	(0.042)
Associates degree	-0.019	-0.056*	-0.017	0.007	-0.018	-0.022	-0.041	0.016	-0.039	0.023	-0.010	0.013	-0.006	-0.076***	0.019
	(0.030)	(0.030)	(0.030)	(0.029)	(0.022)	(0.021)	(0.030)	(0.026)	(0.030)	(0.024)	(0.021)	(0.026)	(0.026)	(0.026)	(0.029)
Bachelor's degree	0.006	-0.029	-0.055**	-0.028	0.020	0.013	0.026	0.033	-0.049**	0.021	0.001	-0.020	-0.036*	-0.051**	0.075***
	(0.025)	(0.025)	(0.025)	(0.025)	(0.018)	(0.017)	(0.025)	(0.022)	(0.025)	(0.021)	(0.017)	(0.022)	(0.022)	(0.022)	(0.024)
Graduate degree	-0.026	-0.051	-0.053*	-0.033	0.005	-0.012	0.067**	0.023	-0.048	0.065**	0.031	-0.003	0.024	-0.013	0.064**
	(0.032)	(0.032)	(0.031)	(0.031)	(0.023)	(0.022)	(0.032)	(0.028)	(0.032)	(0.026)	(0.022)	(0.028)	(0.027)	(0.028)	(0.031)
Black	-0.008	0.008	-0.022	-0.007	0.045**	0.037*	0.057**	-0.064***	-0.004	0.041*	0.018	0.086***	0.002	0.129***	-0.160***
	(0.028)	(0.028)	(0.028)	(0.027)	(0.020)	(0.019)	(0.028)	(0.024)	(0.028)	(0.023)	(0.019)	(0.025)	(0.024)	(0.024)	(0.027)
Hispanic	0.053**	0.027	-0.047*	-0.007	-0.002	-0.008	0.016	-0.025	-0.022	0.046**	0.041**	0.068***	0.059***	-0.018	-0.009
	(0.024)	(0.025)	(0.024)	(0.024)	(0.018)	(0.017)	(0.024)	(0.021)	(0.024)	(0.020)	(0.017)	(0.021)	(0.021)	(0.021)	(0.023)

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Asian	0.005	-0.013	-0.030	-0.095**	-0.007	0.052**	0.179***	0.025	-0.085**	-0.016	-0.025	0.003	0.012	-0.097***	0.018
	(0.038)	(0.038)	(0.038)	(0.037)	(0.027)	(0.026)	(0.038)	(0.033)	(0.038)	(0.031)	(0.027)	(0.034)	(0.033)	(0.033)	(0.037)
Other race	0.143**	0.204***	0.069	0.040	-0.008	-0.016	0.099	-0.049	-0.006	0.129**	-0.039	0.012	0.101*	0.233***	-0.026
	(0.061)	(0.062)	(0.061)	(0.060)	(0.044)	(0.043)	(0.061)	(0.054)	(0.061)	(0.051)	(0.043)	(0.054)	(0.055)	(0.053)	(0.059)
Age 18-29	-0.043	-0.035	0.086***	0.137***	-0.090***	-0.162***	0.166***	-0.084***	0.215***	0.263***	0.288***	0.244***	0.263***	0.108***	-0.065**
	(0.031)	(0.032)	(0.031)	(0.031)	(0.023)	(0.022)	(0.031)	(0.027)	(0.031)	(0.026)	(0.022)	(0.027)	(0.027)	(0.027)	(0.030)
Age 30-49	-0.065**	-0.044	0.025	0.102***	-0.081***	-0.108***	0.151***	-0.065***	0.104***	0.195***	0.228***	0.201***	0.158***	0.141***	-0.144***
	(0.026)	(0.027)	(0.026)	(0.026)	(0.019)	(0.018)	(0.027)	(0.023)	(0.026)	(0.022)	(0.019)	(0.023)	(0.023)	(0.023)	(0.026)
Age 50-64	-0.095***	-0.093***	-0.039	-0.008	-0.056***	-0.056***	0.036	-0.070***	-0.008	0.064***	0.040**	0.056**	0.046**	0.032	-0.140***
	(0.025)	(0.025)	(0.025)	(0.025)	(0.018)	(0.017)	(0.025)	(0.022)	(0.025)	(0.021)	(0.017)	(0.022)	(0.022)	(0.022)	(0.024)
Male	0.045***	0.029*	0.036**	0.045***	-0.006	-0.032***	0.008	-0.042***	0.047***	0.071***	0.071***	0.086***	0.047***	0.040***	0.159***
	(0.017)	(0.018)	(0.017)	(0.017)	(0.013)	(0.012)	(0.017)	(0.015)	(0.017)	(0.014)	(0.012)	(0.015)	(0.015)	(0.015)	(0.017)
I have a high-risk medical condition	0.071***	0.046**	0.020	0.004	0.039***	-0.016	0.115***	0.061***	0.028	0.046***	0.079***	0.051***	0.045**	0.024	0.128***
	(0.020)	(0.020)	(0.020)	(0.020)	(0.015)	(0.014)	(0.020)	(0.018)	(0.020)	(0.017)	(0.014)	(0.018)	(0.017)	(0.017)	(0.019)
Family member has a high-risk medical condition	0.045*	0.033	-0.015	-0.038*	0.056***	0.018	0.073***	0.061***	-0.040*	-0.045**	-0.009	0.008	-0.043**	-0.061***	0.062***
	(0.023)	(0.024)	(0.023)	(0.023)	(0.017)	(0.016)	(0.023)	(0.020)	(0.023)	(0.019)	(0.016)	(0.021)	(0.020)	(0.020)	(0.022)
I and family member have a high-risk medical condition	-0.032	-0.078**	-0.100***	-0.076***	0.056***	0.039*	0.142***	0.063**	-0.058*	-0.053**	0.038*	-0.024	-0.030	-0.079***	0.098***
	(0.030)	(0.030)	(0.030)	(0.029)	(0.022)	(0.021)	(0.030)	(0.026)	(0.030)	(0.025)	(0.021)	(0.027)	(0.026)	(0.026)	(0.029)
Democrat X Experiment-Show AAP recommendation	0.056	0.015	0.047	0.096	-0.035	-0.123**	-0.135*	-0.027	-0.029	0.006	-0.043	-0.033	0.144**	0.130**	0.032
	(0.070)	(0.071)	(0.069)	(0.069)	(0.051)	(0.048)	(0.070)	(0.061)	(0.069)	(0.057)	(0.049)	(0.062)	(0.060)	(0.060)	(0.067)
Democrat X Experiment-Explain low deaths among children	-0.142**	-0.104	0.035	0.145**	-0.068	-0.152***	-0.167**	-0.012	-0.072	0.067	-0.066	-0.044	0.092	0.049	0.031
	(0.069)	(0.070)	(0.068)	(0.068)	(0.050)	(0.048)	(0.069)	(0.061)	(0.069)	(0.057)	(0.048)	(0.061)	(0.060)	(0.060)	(0.067)
Democrat X Experiment-Explain trend in deaths	-0.028	0.028	0.077	0.059	-0.072	-0.121**	-0.056	0.047	-0.010	0.004	-0.042	-0.004	0.046	0.025	0.035
	(0.070)	(0.071)	(0.069)	(0.069)	(0.051)	(0.048)	(0.070)	(0.061)	(0.070)	(0.058)	(0.049)	(0.062)	(0.060)	(0.061)	(0.068)
Democrat X Experiment-Schools are safe	0.055	0.025	0.076	0.062	-0.002	-0.121**	-0.066	-0.005	0.008	-0.026	-0.014	-0.062	0.066	0.021	0.088

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	(0.070)	(0.071)	(0.069)	(0.069)	(0.051)	(0.049)	(0.070)	(0.062)	(0.070)	(0.058)	(0.049)	(0.062)	(0.061)	(0.061)	(0.068)
Democrat X Experiment-Case showing masks prevent infections at salon	-0.004	-0.067	-0.022	0.051	-0.024	-0.148***	0.001	0.087	-0.100	0.015	-0.005	-0.060	0.079	0.123**	0.048
	(0.071)	(0.073)	(0.071)	(0.071)	(0.052)	(0.050)	(0.072)	(0.063)	(0.071)	(0.059)	(0.050)	(0.063)	(0.062)	(0.062)	(0.069)
Democrat X Experiment-Case showing flights are safe	0.023	-0.023	0.046	0.086	-0.030	-0.091*	0.044	0.071	-0.048	-0.001	-0.037	-0.023	0.028	0.124**	0.066
	(0.071)	(0.072)	(0.071)	(0.070)	(0.052)	(0.049)	(0.071)	(0.063)	(0.071)	(0.059)	(0.050)	(0.063)	(0.062)	(0.062)	(0.069)
Democrat X Experiment-Hospitalizations are rising	0.087	0.076	0.119*	0.149**	0.076	-0.028	-0.077	-0.041	-0.015	0.039	-0.045	0.003	0.017	0.055	0.014
	(0.070)	(0.071)	(0.070)	(0.069)	(0.051)	(0.049)	(0.070)	(0.062)	(0.070)	(0.058)	(0.049)	(0.062)	(0.061)	(0.061)	(0.068)
Democrat X Experiment-Current unemployment rate	0.132*	0.048	0.090	0.114*	-0.024	-0.098**	-0.117*	0.009	-0.020	-0.047	-0.039	-0.049	0.118**	0.125**	0.068
	(0.069)	(0.071)	(0.069)	(0.068)	(0.050)	(0.048)	(0.069)	(0.061)	(0.069)	(0.057)	(0.049)	(0.061)	(0.060)	(0.060)	(0.067)
Constant	0.437***	0.432***	0.307***	0.159***	0.940***	1.032***	0.512***	0.783***	0.470***	0.022	-0.083**	0.084*	0.341***	0.435***	0.632***
	(0.053)	(0.055)	(0.053)	(0.053)	(0.039)	(0.037)	(0.054)	(0.047)	(0.056)	(0.047)	(0.040)	(0.050)	(0.049)	(0.049)	(0.052)
Observations	3,286	3,286	3,286	3,286	3,286	3,286	3,286	3,286	3,287	3,287	3,287	3,287	3,279	3,279	3,279
Adjusted R-squared	0.078	0.079	0.144	0.135	0.048	0.066	0.126	0.077	0.136	0.184	0.239	0.155	0.150	0.095	0.165

Robust standard errors in parentheses, *** p<0.01, ** p<0.05, * p<0.1

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Supplemental Table 5. Regress experimental condition on all outcomes with media diet interactions, controlling for full set of demographic variables. Sample restricted to those with left-leaning or right-leaning diet

	(1) Support in-person schools	(2) Support In-person school college	(3) Support Indoor dining at restaurants	(4) Support Indoor seating at bars	(5) Support requiring masks in indoor public spaces	(6) Support requiring that workers wear masks with others	(7) Support closing non-essential business temp close in-person	(8) Support prohibiting large indoor gatherings	(9) Behavior-Likely to eat out	(10) Behavior-Likely to book personal flight	(11) Behavior-Likely to book business flight	(12) Behavior-Likely to book hotel	(13) Believe COVID is under control in your state	(14) Very confident can protect oneself	(15) Agree to effective COVID vaccine in Spring
Experiment-Show AAP recommendation	0.137*	0.120	0.001	-0.075	0.044	0.045	0.092	0.103*	0.033	0.040	0.005	0.073	-0.126**	-0.058	0.060
	(0.073)	(0.074)	(0.070)	(0.068)	(0.052)	(0.047)	(0.072)	(0.060)	(0.070)	(0.056)	(0.046)	(0.060)	(0.058)	(0.058)	(0.069)
Experiment-Explain low deaths among children	-0.051	-0.093	-0.090	-0.213***	0.086	0.103**	0.156**	0.202***	-0.010	-0.042	-0.116**	0.042	-0.016	-0.066	0.018
	(0.077)	(0.078)	(0.074)	(0.072)	(0.054)	(0.050)	(0.076)	(0.064)	(0.074)	(0.060)	(0.049)	(0.064)	(0.061)	(0.062)	(0.073)
Experiment-Explain trend in deaths	-0.012	-0.014	0.068	-0.066	-0.005	0.069	0.041	0.008	0.133*	0.143**	-0.066	0.133**	-0.041	-0.055	-0.165**
	(0.081)	(0.082)	(0.078)	(0.076)	(0.057)	(0.052)	(0.079)	(0.067)	(0.077)	(0.063)	(0.051)	(0.067)	(0.064)	(0.065)	(0.076)
Experiment-Schools are safe	0.168**	0.073	-0.022	0.052	0.110**	0.140***	0.052	0.110*	0.067	0.053	-0.031	0.052	-0.227***	-0.113*	-0.004
	(0.074)	(0.074)	(0.071)	(0.069)	(0.052)	(0.047)	(0.072)	(0.061)	(0.070)	(0.057)	(0.046)	(0.060)	(0.058)	(0.059)	(0.069)
Experiment-Case showing masks prevent infections at salon	0.041	0.007	0.013	-0.088	-0.021	0.065	0.023	0.047	-0.015	0.062	-0.109**	0.025	-0.151**	0.005	-0.046
	(0.077)	(0.078)	(0.075)	(0.072)	(0.055)	(0.050)	(0.076)	(0.064)	(0.074)	(0.060)	(0.049)	(0.064)	(0.061)	(0.062)	(0.073)
Experiment-Case showing flights are safe	-0.064	-0.069	-0.051	-0.152**	0.015	0.113**	0.036	0.127**	-0.051	0.025	-0.052	0.002	-0.051	-0.133**	0.009
	(0.076)	(0.076)	(0.073)	(0.071)	(0.053)	(0.049)	(0.074)	(0.063)	(0.072)	(0.058)	(0.048)	(0.062)	(0.059)	(0.061)	(0.071)
Experiment-Hospitalizations are rising	0.090	0.086	-0.016	-0.055	-0.030	0.113**	0.151**	0.083	0.069	0.083	-0.013	0.069	-0.113*	-0.093	-0.105
	(0.078)	(0.078)	(0.075)	(0.073)	(0.055)	(0.050)	(0.076)	(0.064)	(0.074)	(0.060)	(0.049)	(0.064)	(0.061)	(0.062)	(0.073)
Experiment-Current unemployment rate	0.021	0.023	-0.026	-0.132*	-0.072	-0.002	0.048	0.027	-0.035	0.084	0.005	0.105	-0.047	-0.032	0.060
	(0.078)	(0.078)	(0.075)	(0.073)	(0.055)	(0.050)	(0.076)	(0.064)	(0.074)	(0.060)	(0.049)	(0.064)	(0.061)	(0.062)	(0.073)
Annual flights for work in 2019	0.006**	0.003	0.002	0.004*	-0.001	0.002	0.008***	0.003*	0.005**	0.006***	0.007***	0.005***	0.008***	0.003*	0.006***
	(0.002)	(0.002)	(0.002)	(0.002)	(0.002)	(0.001)	(0.002)	(0.002)	(0.002)	(0.002)	(0.001)	(0.002)	(0.002)	(0.002)	(0.002)
Annual flights for pleasure in 2019	0.001	0.001	0.003	0.005*	-0.003*	-0.009***	-0.004	-0.004*	0.007***	0.012***	0.005***	0.011***	0.008***	0.003	0.003
	(0.003)	(0.003)	(0.002)	(0.002)	(0.002)	(0.002)	(0.003)	(0.002)	(0.002)	(0.002)	(0.002)	(0.002)	(0.002)	(0.002)	(0.002)
Numeracy--number correct on 3-item test	-0.056***	-0.044***	-0.047***	-0.042***	0.024***	0.023***	0.001	0.027**	-0.040***	-0.044***	-0.058***	-0.048***	-0.052***	-0.044***	0.035***
	(0.013)	(0.013)	(0.013)	(0.012)	(0.009)	(0.009)	(0.013)	(0.011)	(0.013)	(0.010)	(0.008)	(0.011)	(0.010)	(0.011)	(0.013)

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Currently invested in stock market	0.047***	0.009	0.037*	0.004	0.056***	0.054***	-0.006	0.057***	0.013	0.031*	0.049***	0.040**	0.026	-0.001	0.101***
	(0.022)	(0.022)	(0.021)	(0.021)	(0.016)	(0.014)	(0.022)	(0.018)	(0.021)	(0.017)	(0.014)	(0.018)	(0.018)	(0.018)	(0.021)
Employed	0.056***	0.041*	0.066***	0.079***	-0.027*	-0.028*	-0.035	-0.047**	0.103***	0.065***	0.065***	0.054***	0.054***	0.033*	-0.007
	(0.023)	(0.023)	(0.022)	(0.021)	(0.016)	(0.014)	(0.022)	(0.019)	(0.022)	(0.017)	(0.014)	(0.019)	(0.018)	(0.018)	(0.021)
Homeowner	-0.035	-0.020	0.017	-0.004	-0.004	-0.018	-0.046**	-0.026	0.020	0.006	-0.007	-0.023	-0.015	-0.002	0.019
	(0.023)	(0.023)	(0.022)	(0.022)	(0.016)	(0.015)	(0.023)	(0.019)	(0.022)	(0.018)	(0.015)	(0.019)	(0.018)	(0.019)	(0.022)
Household income (1-10 categories)	0.010*	0.012**	-0.005	-0.004	-0.001	0.003	-0.001	0.007*	-0.004	-0.002	-0.007**	-0.001	-0.002	-0.005	-0.002
	(0.005)	(0.005)	(0.005)	(0.005)	(0.004)	(0.003)	(0.005)	(0.004)	(0.005)	(0.004)	(0.003)	(0.004)	(0.004)	(0.004)	(0.005)
Pre-COVID household budget-Indebtedness (5) vs savings (1)	-0.023***	-0.021*	-0.024**	-0.018*	-0.005	-0.003	0.005	-0.018**	-0.029***	-0.028***	-0.033***	-0.043***	-0.044***	-0.064***	-0.035***
	(0.011)	(0.011)	(0.010)	(0.010)	(0.008)	(0.007)	(0.011)	(0.009)	(0.010)	(0.008)	(0.007)	(0.009)	(0.009)	(0.009)	(0.010)
Democrat	-0.008	-0.020	-0.066***	-0.060***	0.015	0.032**	0.128***	0.064***	-0.013	0.006	-0.014	0.016	-0.008	-0.000	0.165***
	(0.023)	(0.023)	(0.022)	(0.022)	(0.016)	(0.015)	(0.023)	(0.019)	(0.022)	(0.018)	(0.015)	(0.019)	(0.018)	(0.019)	(0.022)
Republican	0.166***	0.194***	0.257***	0.208***	-0.048**	-0.041**	-0.032	-0.113***	0.213***	0.108***	0.073***	0.133***	0.156***	0.101***	-0.011
	(0.027)	(0.027)	(0.026)	(0.025)	(0.019)	(0.017)	(0.026)	(0.022)	(0.025)	(0.021)	(0.017)	(0.022)	(0.021)	(0.021)	(0.025)
Less than high school	0.089	0.066	0.014	0.113*	-0.035	-0.047	0.124**	0.014	0.151**	0.054	0.105***	0.066	0.114**	-0.011	0.003
	(0.062)	(0.062)	(0.059)	(0.058)	(0.043)	(0.040)	(0.060)	(0.051)	(0.059)	(0.048)	(0.039)	(0.051)	(0.049)	(0.050)	(0.058)
Some college without degree	0.014	0.000	0.029	0.056**	0.024	-0.018	-0.003	0.013	0.050*	0.066***	0.024	0.042*	0.016	-0.005	0.029
	(0.028)	(0.028)	(0.027)	(0.026)	(0.020)	(0.018)	(0.027)	(0.023)	(0.027)	(0.022)	(0.018)	(0.023)	(0.022)	(0.023)	(0.026)
Industry certification or certificate	0.042	0.040	0.041	0.117**	-0.027	-0.033	-0.079	0.018	0.109**	0.056	0.096***	0.071*	0.008	-0.031	0.009
	(0.049)	(0.050)	(0.047)	(0.046)	(0.035)	(0.032)	(0.048)	(0.041)	(0.047)	(0.038)	(0.031)	(0.041)	(0.039)	(0.040)	(0.046)
Associates degree	-0.015	-0.030	0.004	0.000	0.015	0.011	-0.073**	0.035	-0.028	0.011	-0.030	-0.026	-0.013	-0.070**	0.005
	(0.036)	(0.036)	(0.035)	(0.034)	(0.025)	(0.023)	(0.035)	(0.030)	(0.034)	(0.028)	(0.023)	(0.030)	(0.028)	(0.029)	(0.034)
Bachelor's degree	0.019	-0.012	-0.025	-0.009	0.018	0.009	-0.010	-0.002	0.006	0.049**	0.018	0.003	0.000	-0.041*	0.080***
	(0.029)	(0.029)	(0.028)	(0.027)	(0.020)	(0.018)	(0.028)	(0.024)	(0.027)	(0.022)	(0.018)	(0.024)	(0.023)	(0.023)	(0.027)
Graduate degree	-0.044	-0.079**	-0.036	-0.028	0.014	-0.009	0.023	0.026	-0.042	0.098***	0.037	0.015	0.002	-0.039	0.097***
	(0.036)	(0.036)	(0.034)	(0.033)	(0.025)	(0.023)	(0.035)	(0.029)	(0.034)	(0.028)	(0.023)	(0.030)	(0.028)	(0.029)	(0.034)
Black	-0.074***	-0.027	0.012	-0.000	0.036	0.042**	0.123***	-0.015	-0.008	0.031	0.015	0.078***	-0.005	0.116***	-0.169***
	(0.033)	(0.033)	(0.031)	(0.030)	(0.023)	(0.021)	(0.032)	(0.027)	(0.031)	(0.025)	(0.021)	(0.027)	(0.026)	(0.026)	(0.031)
Hispanic	0.027	0.027	-0.065**	-0.044*	-0.010	-0.022	0.043	-0.025	-0.074***	0.048**	0.055***	0.037	0.083***	-0.004	-0.025

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	(0.028)	(0.029)	(0.027)	(0.027)	(0.020)	(0.018)	(0.028)	(0.023)	(0.027)	(0.022)	(0.018)	(0.023)	(0.022)	(0.023)	(0.027)
Asian	-0.006	0.033	-0.027	-0.071*	0.009	0.024	0.127***	0.056	-0.105**	-0.000	-0.035	0.018	0.066*	-0.085**	-0.013
	(0.043)	(0.043)	(0.041)	(0.040)	(0.030)	(0.028)	(0.042)	(0.035)	(0.041)	(0.033)	(0.027)	(0.036)	(0.034)	(0.035)	(0.041)
Other race	0.024	0.143**	0.054	-0.022	0.026	-0.036	0.155**	0.015	0.159**	0.160***	0.088**	0.034	0.028	0.093*	-0.137**
	(0.066)	(0.067)	(0.064)	(0.062)	(0.047)	(0.043)	(0.065)	(0.055)	(0.063)	(0.051)	(0.042)	(0.055)	(0.052)	(0.053)	(0.063)
Age 18-29	-0.091**	-0.006	0.073**	0.086**	-0.043*	-0.103***	0.144***	-0.063**	0.173***	0.175***	0.222***	0.149***	0.124***	0.035	-0.093***
	(0.037)	(0.037)	(0.035)	(0.034)	(0.026)	(0.024)	(0.036)	(0.030)	(0.035)	(0.028)	(0.023)	(0.030)	(0.029)	(0.030)	(0.035)
Age 30-49	-0.110***	-0.041	-0.014	0.056*	-0.042*	-0.056***	0.148***	-0.030	0.053*	0.121***	0.155***	0.136***	0.064***	0.073***	-0.153***
	(0.030)	(0.031)	(0.029)	(0.029)	(0.021)	(0.020)	(0.030)	(0.025)	(0.029)	(0.024)	(0.019)	(0.025)	(0.024)	(0.025)	(0.029)
Age 50-64	-0.078***	-0.054*	-0.036	-0.005	-0.016	-0.010	0.045*	-0.030	-0.022	0.055**	0.031*	0.045*	0.004	0.004	-0.121***
	(0.028)	(0.028)	(0.027)	(0.026)	(0.020)	(0.018)	(0.027)	(0.023)	(0.027)	(0.022)	(0.018)	(0.023)	(0.022)	(0.022)	(0.026)
Male	0.019	0.021	0.033*	0.039**	-0.027*	-0.049***	0.011	-0.076***	0.058***	0.056***	0.076***	0.080***	0.039**	0.052***	0.145***
	(0.020)	(0.020)	(0.019)	(0.018)	(0.014)	(0.013)	(0.019)	(0.016)	(0.019)	(0.015)	(0.013)	(0.016)	(0.016)	(0.016)	(0.019)
I have a high-risk medical condition	0.065***	0.069***	0.025	0.013	0.010	-0.029**	0.115***	0.048**	0.010	0.031*	0.058***	0.064***	0.040**	0.023	0.068***
	(0.023)	(0.023)	(0.022)	(0.022)	(0.016)	(0.015)	(0.023)	(0.019)	(0.022)	(0.018)	(0.015)	(0.019)	(0.018)	(0.019)	(0.022)
Family member has a high-risk medical condition	0.034	0.048*	0.003	0.012	0.048**	0.017	0.089***	0.030	-0.017	0.001	0.047***	0.049**	0.017	-0.018	0.081***
	(0.027)	(0.027)	(0.026)	(0.025)	(0.019)	(0.017)	(0.026)	(0.022)	(0.026)	(0.021)	(0.017)	(0.022)	(0.021)	(0.022)	(0.025)
I and family member have a high-risk medical condition	-0.043	-0.066*	-0.002	-0.030	0.069***	0.044**	0.174***	0.046	-0.039	-0.020	0.034	-0.002	0.011	-0.035	0.055*
	(0.034)	(0.034)	(0.032)	(0.032)	(0.024)	(0.022)	(0.033)	(0.028)	(0.032)	(0.026)	(0.021)	(0.028)	(0.027)	(0.027)	(0.032)
Left-media diet X Experiment-Show AAP recommendation	0.076	-0.009	0.082	0.160**	-0.066	-0.062	-0.185**	-0.085	0.079	-0.018	0.032	-0.045	0.170**	0.062	-0.124
	(0.087)	(0.087)	(0.084)	(0.081)	(0.061)	(0.056)	(0.085)	(0.072)	(0.083)	(0.067)	(0.055)	(0.071)	(0.068)	(0.069)	(0.081)
Left-media diet X Experiment-Explain low deaths among children	0.166*	0.191**	0.142*	0.267***	-0.131**	-0.115**	-0.214**	-0.159**	0.098	0.094	0.218***	0.001	0.084	0.069	-0.009
	(0.089)	(0.090)	(0.086)	(0.083)	(0.063)	(0.057)	(0.087)	(0.074)	(0.085)	(0.069)	(0.056)	(0.073)	(0.070)	(0.071)	(0.084)
Left-media diet X Experiment-Explain trend in deaths	0.108	0.118	0.044	0.154*	-0.012	-0.055	-0.100	-0.013	-0.041	-0.113	0.130**	-0.090	0.042	0.088	0.162*
	(0.092)	(0.093)	(0.089)	(0.086)	(0.065)	(0.060)	(0.091)	(0.076)	(0.088)	(0.071)	(0.058)	(0.076)	(0.073)	(0.074)	(0.087)

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Left-media diet X Experiment-Schools are safe	-0.012	-0.036	0.054	-0.037	-0.158**	-0.165***	-0.178**	-0.066	0.023	0.008	0.085	0.020	0.228***	0.133*	0.018
	(0.087)	(0.088)	(0.084)	(0.082)	(0.061)	(0.056)	(0.086)	(0.072)	(0.083)	(0.067)	(0.055)	(0.072)	(0.069)	(0.070)	(0.082)
Left-media diet X Experiment-Case showing masks prevent infections at salon	0.084	0.053	0.075	0.136	-0.005	-0.078	-0.122	-0.048	0.051	-0.055	0.197***	0.001	0.208***	0.073	-0.031
	(0.091)	(0.092)	(0.087)	(0.085)	(0.064)	(0.058)	(0.089)	(0.075)	(0.087)	(0.070)	(0.057)	(0.075)	(0.071)	(0.073)	(0.086)
Left-media diet X Experiment-Case showing flights are safe	0.176**	0.169*	0.118	0.209**	-0.035	-0.108*	-0.091	-0.121	0.115	0.054	0.144**	0.113	0.062	0.159**	0.014
	(0.089)	(0.089)	(0.085)	(0.083)	(0.063)	(0.057)	(0.087)	(0.073)	(0.085)	(0.068)	(0.056)	(0.073)	(0.070)	(0.071)	(0.083)
Left-media diet X Experiment-Hospitalizations are rising	0.001	-0.096	0.070	0.121	-0.012	-0.153***	-0.143	-0.007	-0.006	-0.018	0.089	0.020	0.141**	0.147**	0.093
	(0.090)	(0.091)	(0.087)	(0.084)	(0.064)	(0.058)	(0.088)	(0.075)	(0.086)	(0.070)	(0.057)	(0.074)	(0.071)	(0.072)	(0.085)
Left-media diet X Experiment-Current unemployment rate	0.111	0.041	0.084	0.189**	0.083	0.031	-0.066	0.018	0.129	-0.043	0.094*	-0.031	0.057	0.126*	-0.007
	(0.090)	(0.091)	(0.087)	(0.084)	(0.063)	(0.058)	(0.088)	(0.074)	(0.086)	(0.069)	(0.057)	(0.074)	(0.071)	(0.072)	(0.085)
Diet==Left	-0.148**	-0.178***	-0.270***	-0.327***	0.171***	0.205***	0.296***	0.212***	-0.211***	-0.024	-0.085**	-0.032	-0.254***	-0.186***	0.110*
	(0.065)	(0.065)	(0.062)	(0.060)	(0.046)	(0.042)	(0.063)	(0.053)	(0.061)	(0.050)	(0.041)	(0.053)	(0.050)	(0.051)	(0.060)
Constant	0.588***	0.554***	0.582***	0.523***	0.761***	0.771***	0.167**	0.630***	0.396***	-0.025	0.020	0.041	0.345***	0.419***	0.412***
	(0.075)	(0.076)	(0.072)	(0.070)	(0.053)	(0.048)	(0.074)	(0.062)	(0.071)	(0.058)	(0.047)	(0.062)	(0.059)	(0.060)	(0.070)
Observations	2,601	2,601	2,601	2,601	2,601	2,601	2,601	2,601	2,603	2,603	2,603	2,603	2,600	2,595	2,599
Adjusted R-squared	0.083	0.090	0.163	0.157	0.069	0.098	0.137	0.113	0.149	0.137	0.201	0.128	0.161	0.091	0.181

Robust standard errors in parentheses, *** p<0.01, ** p<0.05, * p<0.1

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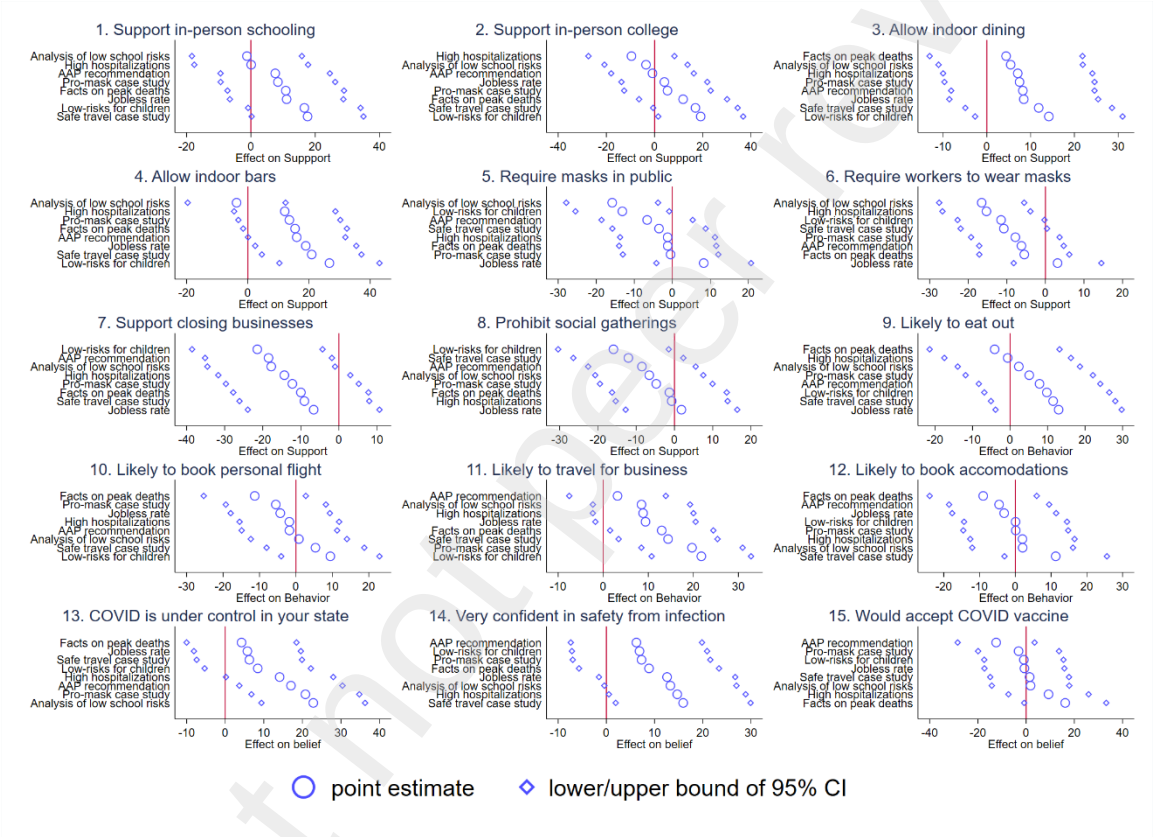
Supplemental Figure 2. Treatment effects for those with a Left-leaning Diet Relative to those with a Right-leaning Diet. Each figure plots treatment effects for respondents identified as consuming a left-leaning news diet relative to a right-leaning diet. Effects are identified in a model that excludes those with a mixed-media diet and is in relation to the experimental reference group. Models 1-8 are policy preferences. Models 9-12 are consumer behaviors, and 13-15 are beliefs. All models control for the demographic characteristics listed in text, including: age, education, race, employment status, political affiliation, numeracy, medical risk factors, household income, health status, and gender. 95% confidence intervals are shown.



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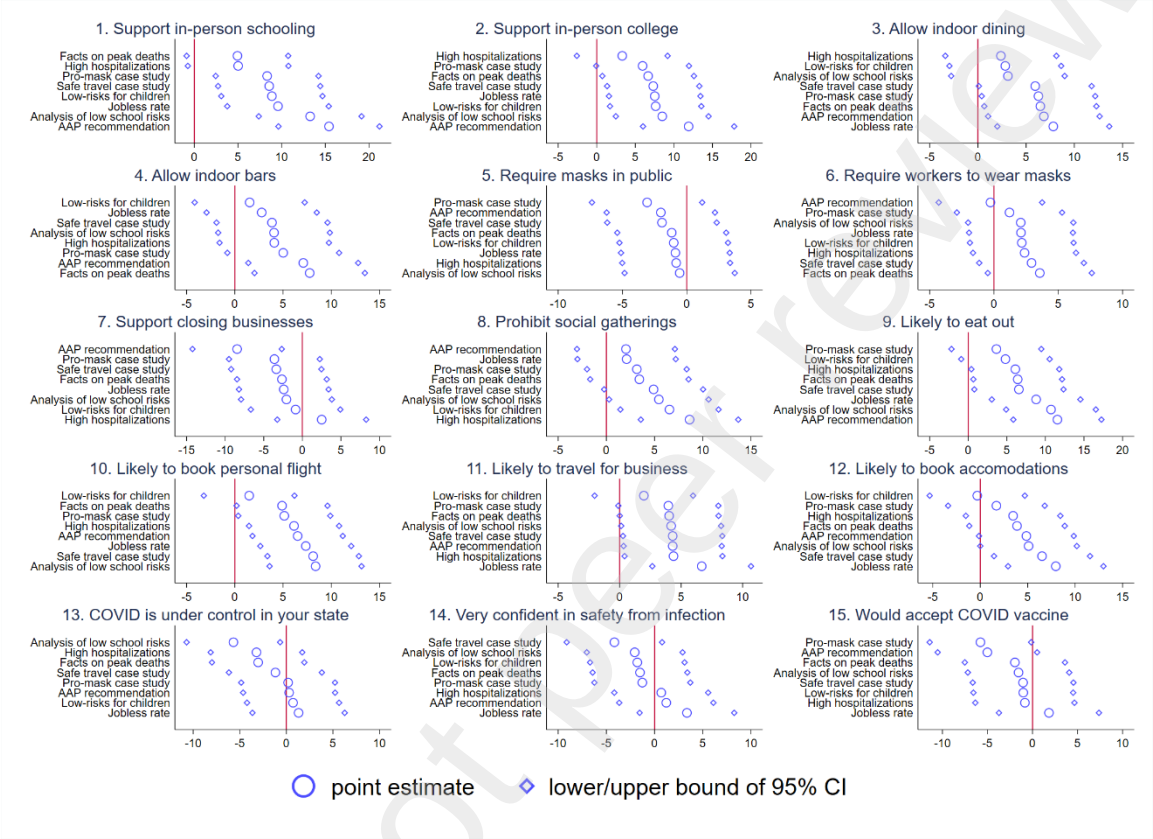
Supplemental Figure 3. Treatment effects in baseline model with no adjustments. Each figure plots the coefficients from the baseline model associated with 8 treatments relative to the reference group. Models 1-8 are policy preferences. Models 9-12 are consumer behaviors, and 13-15 are beliefs. No control variables are included. 95% confidence intervals are shown.



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